

Testing

Screening Testing **NEW AND UPDATED**

NEW Q: What is screening testing?

A: [Screening tests](#) are intended to identify infection with SARS-CoV-2, the virus that causes COVID-19, in unvaccinated people who are asymptomatic and do not have known, suspected, or reported exposure to SARS-CoV-2. Screening helps to identify unknown cases so that measures can be taken to prevent further transmission. Screening testing is separate and distinct from diagnostic testing, which should be performed on anyone that has signs and symptoms consistent with COVID-19 or following recent known or suspected exposure to SARS-CoV-2.

UPDATED Q: Should agencies establish a screening testing program for SARS-CoV-2, the virus that causes COVID-19?

A: Yes, agencies should establish a COVID-19 screening testing program by February 15, 2022 for employees who are not fully vaccinated, including due to a pending or approved request for exception or extension from the COVID-19 vaccination requirement for Federal employees. If an agency is engaged in enforcement steps with an employee covered by the vaccination requirement who is not fully vaccinated and who does not have a pending or approved exception or extension request, those employees should also be included in the agency's screening testing program.

An agency generally does not need to include onsite contractor employees or fully vaccinated employees in its screening testing program. For certain roles, functions, or work environments, an agency may determine that it is necessary that certain onsite contractor employees, certain employees regardless of their vaccination status, or certain employees and certain onsite contractor employees regardless of their vaccination status must participate in screening testing, given operational or administrative considerations associated with conducting screening testing for those roles, functions, or work environments.

NEW Q: What testing protocols should be applied through an agency's screening testing program to employees who are not fully vaccinated?

A: Employees who are not fully vaccinated and who work onsite should undergo regular screening testing through their agency's screening testing program. This includes employees who do not work at a Federal facility but who interact in person with members of the public as part of their job duties, such as safety inspectors.

For purposes of regularly testing these employees through a screening testing program, an employee enrolled in the program should be tested weekly for any week during which they work onsite or interact in person with members of the public as part of their job duties. Agencies may require more frequent testing, such as for certain roles, functions, or work environments. Testing may be conducted at a Federal facility or offsite as determined by the agency. The test should not be both self-administered and self-read by the employee unless observed by the agency or an authorized telehealth provider.

NEW Q: Do Federal employees or contractor employees participating in an agency screening testing program need to be able to provide the results of a negative test in order to enter or be present in a Federal facility?

Federal employees participating in an agency screening testing program do not need to be able to provide the results of a negative test each time they enter or are present in their agency's facilities, unless required as part of the agency's screening testing program and workplace safety protocols. That said, Federal employees generally are treated as visitors during a visit to another agency, so while they are onsite at *another* agency's worksite, they should be able to show proof of a negative COVID-19 test result administered within the past 3 days, if they are not fully vaccinated or decline to provide their vaccination information to the host agency.

Similarly, contractor employees participating in an agency testing program do not need to be able to provide the results of a negative test each time they enter or are present at that agency's facilities, unless required as part of the agency's testing program and workplace safety protocols. However, if they are visiting *another* agency's facilities, they should be able to show proof of a negative COVID-19 test result administered within the past 3 days, if they are not fully vaccinated or decline to provide their vaccination information to the host agency.

NEW Q: Do the screening testing protocols apply to employees who are not fully vaccinated and who are not reporting to their worksite (e.g., are on maximum telework, working remotely, or on leave)?

A: No, employees who are working remotely or who are on maximum telework do not need to undergo regular testing. An employee who does not report to a worksite or interact in person with members of the public as part of their job duties during a particular week (e.g., due to taking leave, teleworking, or working remotely) does not need to be tested that week.

Should these employees have a one-time need to report to one of their agency's facilities, they should be tested in the week they are entering the facility. Should these employees need to be onsite at another agency's facility, they generally would be treated as visitors during their visit, and so should be able to show proof of a negative COVID-19 test result administered within the past 3 days.

NEW Q: Do the screening testing protocols apply to Federal employees who are not fully vaccinated and who only work onsite on an infrequent basis?

A: Federal employees who are not fully vaccinated and who work onsite or interact in person with members of the public as part of their job duties on an infrequent basis should be tested at least once in the week that they are working onsite or interact in person with members of the public as part of their job duties. An agency may determine that some employees who generally do not report to a worksite or interact with members of the public as part of their job duties, but may need to do so on an emergency basis (e.g., for accident investigations or disaster response) are required to undergo regular testing on a frequency determined by the agency.

Should these Federal employees need to be onsite at another agency's facility, they generally would be treated as visitors during their visit, and so would need to be able to show proof of a negative COVID-19 test result administered within the past 3 days.

UPDATED Q: What types of tests may an agency utilize for a screening testing program?

A: For screening testing programs, agencies may utilize any viral tests [authorized by the Food and Drug Administration](#) (FDA) to detect current infection. There are two types of [viral tests](#): [antigen tests](#) and [nucleic acid application tests \(NAATs\)](#). Reverse transcriptase polymerase chain reaction (RT-PCR) tests are a type of NAAT. Agencies should consult [FDA guidance](#) for specific authorized tests impacted by COVID-19 variants and the current status on recommended use.

Options for testing methods include: (1) in-store or drive-through point-of-care (POC) testing, such as at pharmacies, (2) swab-testing capabilities that enable an individual to collect the specimen—using a self-collection kit—and drop it off at a designated collection location or ship it to a laboratory, (3) over-the-counter (OTC) tests, as long as those tests are not both self-

administered and self-read by the employee unless observed by the agency or an authorized telehealth provider, or (4) other self-administered tests, as long as those tests are not also self-read by the employee unless observed by the agency or an authorized telehealth provider.

Agencies should establish a means of verifying the date and result of a test—the test should not be both self-administered and self-read by the employee unless observed by the agency or an authorized onsite or telehealth provider. If the results are provided to the Federal employee or contractor employee who was tested, the agency should establish a means for the Federal employee or contractor employee to provide those results to the appropriate agency staff for verification that required testing has been completed and the employee has tested negative. Agencies must comply with applicable privacy safeguards and regulations when receiving test results.

UPDATED Q: What steps may an agency take if a Federal employee refuses to take a COVID-19 test required pursuant to that program?

A: Refusals to take a required test or to provide the results of the test may result in disciplinary measures. In addition to pursuing any disciplinary action, an agency may separately elect to bar the employee from the agency workplace for the safety of others pending resolution of any disciplinary or other action the agency may pursue. Any decision to bar the employee should occur in consultation with the agency's onsite security authority, agency's human resources office, and agency's legal counsel. If the agency bars the employee from the workplace, and the nature of the employee's work does not allow for it to be performed outside of the workplace, the employee must be placed on paid administrative leave until the question of disciplinary action is resolved. In pursuing an adverse action, the agency must also follow normal processes to provide the required notice to the employee.

An agency follows a different process if the employee seeks an accommodation from the requirement to be regularly tested. In that case, an agency should follow its ordinary process to review and consider what, if any, accommodation should be offered in accordance with Federal employment nondiscrimination laws. All agency personnel designated to receive requests for disability accommodations should also know how to handle requests consistent with other Federal employment nondiscrimination laws that may apply. While the request is being resolved, the agency may bar the employee from official worksites. During that temporary period, the agency may direct the employee to work from home. If the employee's duties cannot be performed via telework, the employee should be granted administrative leave for the temporary period of time while the request is being resolved.

If the employee's request for an accommodation to a testing requirement is denied, and the employee does not comply with the testing requirement, the agency may pursue disciplinary

action up to and including removal.

UPDATED Q: Are agencies required to pay for the cost of testing employees pursuant to a screening testing program?

A: Yes, agencies are required to pay for the cost of screening testing of Federal employees pursuant to the agency's screening testing program. Agencies are also responsible for paying the cost of required testing should an employee visit another Federal agency if the employee's agency has approved the visit in advance. Agencies can either pay for the testing directly or reimburse employees for required tests. Should agencies have questions regarding their approach to funding this testing from agency appropriations, they should contact their respective OMB Resource Management Office.

UPDATED Q: Can agencies utilize pooled specimen testing in a screening testing program?

A: Yes, agencies can utilize [pooled specimen testing](#), assuming the screening testing program utilizes any COVID-19 viral test that is used to detect current infection and has been [authorized by the Food and Drug Administration](#) for use in pooled testing. Agencies should consult [FDA guidance](#) for specific authorized tests impacted by COVID-19 variants and the current status on recommended use.

Pooled specimen testing combines the same type of specimen from several people and conducts one NAAT laboratory test on the combined specimen. If the pooled specimen test returns a positive result, each specimen in the pool must be retested individually. Of note, as the incidence of COVID-19 increases among the population being tested, the cost savings of a pooling strategy decreases because more pooled tests will return positive results, and those specimens will need to be retested individually to determine which individual(s) are positive.

UPDATED Q: Is required screening testing undertaken on duty time?

A: Yes. When a Federal employee is required to be tested pursuant to an agency's screening testing program, the time the employee spends obtaining the test (including travel time) from a site preapproved by the agency is duty time; thus, there is no need for the employee to take administrative leave for such time during the employee's basic tour of duty. An agency should only authorize an employee to spend time related to screening testing during the employee's basic tour of duty hours and only for the amount of time necessary. Agencies should determine the amount of time to authorize employees to take to travel to the testing site (if travel is required), complete testing, and return to work. Agencies should require employees taking longer than that amount of time to document the reasons for the additional time. If, due to

unforeseen circumstances, the employee is unable to obtain the test during basic tour of duty hours, the normal overtime hours of work rules apply.

Reasonable travel costs that are incurred as a result of obtaining the test from a site preapproved by the agency should be handled the same way as local travel or temporary duty (TDY) cost reimbursement is handled based on agency policy and the Federal Travel Regulation.

When an employee is not required to be tested but decides to obtain a COVID-19 test during basic tour of duty hours at the employee's own initiative, the employee may request sick leave or other available paid time off for that situation.

UPDATED Q: Are Federal employees and contractor employees participating in an agency screening testing program limited in their ability to work onsite in between tests?

A: No, provided that they have met any applicable testing requirement and have not tested positive for COVID-19, Federal employees and contractor employees participating in an agency screening testing program are not limited in their ability to work onsite between tests, although they should comply with all relevant safety protocols. However, if the employee or contractor employee has come into [close contact](#) with a person with COVID-19 during this time, they should follow CDC guidelines for testing and quarantine, as well as any other requirements set forth in agency workplace safety protocols. Similarly, if they have symptoms consistent with COVID-19, they should follow agency protocols and not enter a worksite.

Agencies should develop a procedure for addressing circumstances in which employees or onsite contractor employees miss their required test, which may include restricting the individual's access to worksites if they have not obtained a test within a period of time specified by the agency.

An employee's failure to comply with testing requirements can result in disciplinary action, up to and including removal. An agency may separately elect to bar the employee from the agency workplace for the safety of others pending resolution of any disciplinary action. A contractor employee's failure to comply with testing requirements may result in that individual being denied entry to a Federal facility. Such circumstances do not relieve the contractor from meeting all contractual requirements.

Diagnostic Testing NEW AND UPDATED

Testing Results and Information UPDATED

Testing and Labor Relations

Find COVID-19
Vaccines Near You

Visit [Vaccines.gov](https://www.vaccines.gov)

Or Call 1-800-232-0233



For questions or comments, email the Safer Federal Workforce Task Force at SaferFederalWorkforce@gsa.gov





COVID-19

Testing Strategies for SARS-CoV-2

Updated Dec. 28, 2021

CDC has updated [isolation and quarantine](#) recommendations for the public, and is revising the CDC website to reflect these changes. These recommendations do not apply to [healthcare personnel](#) and do not supersede state, local, tribal, or territorial laws, rules, and regulations.

Summary of Recent Changes

Updates as of August 13, 2021



- Updated information for fully vaccinated people given new evidence on the B.1.617.2 (Delta) variant currently circulating in the United States.

Key Points

- This guidance describes and compares different types of testing strategies SARS-CoV-2 (the virus that causes COVID-19), including their intended use and applications, regulatory requirements, and reporting requirements.
- This guidance is intended for those who offer and perform SARS-CoV-2 testing.

Diagnostic Testing

Diagnostic testing is intended to identify current infection in individuals and should be performed on anyone that has signs and symptoms consistent with COVID-19 and/or following recent known or suspected exposure to SARS-CoV-2.

Examples of diagnostic testing include:

- Testing anyone with symptoms consistent with COVID-19
- Testing vaccinated and unvaccinated people who were exposed to someone with a confirmed or suspected case of COVID-19

Screening Testing

Screening tests are intended to identify unvaccinated people with COVID-19 who are asymptomatic and do not have known, suspected, or reported exposure to SARS-CoV-2. Screening helps to identify unknown cases so that measures can be taken to prevent further transmission.

Examples of screening include testing:

- Employees in a workplace setting
- Students, faculty, and staff in a school setting
- A person before or after travel
- Someone at home who does not have symptoms associated with COVID-19 and no known exposures to someone with COVID-19

Public Health Surveillance Testing

Public health surveillance is the ongoing, systematic collection, analysis, and interpretation of health-related data essential to the planning, implementation, and evaluation of public health practice. See CDC's [Introduction to Public Health Surveillance](#).

Public health surveillance testing is intended to monitor community- or population-level outbreaks of disease, or to characterize the incidence and prevalence of disease. Surveillance testing is performed on de-identified specimens, and thus, results are not linked to individual people. Public health surveillance testing results cannot be used for individual decision-making.

Public health surveillance testing may sample a certain percentage of a specific population to monitor for increasing or decreasing prevalence, or to determine the population effect from community interventions such as social distancing. An example of public health surveillance testing is when a state public health department develops a plan to randomly select and sample a percentage of all people in a city on a rolling basis to assess local infection rates and trends.

Regulatory Requirements for Diagnostic, Screening, and Public Health Surveillance Testing

Any laboratory or testing site that performs **diagnostic** or **screening** testing must have a Clinical Laboratory Improvement Amendments (CLIA) certificate and meet all applicable CLIA requirements. For more information, see the Centers for Medicare & Medicaid Services [CLIA website](#) . Tests used for SARS-CoV-2 diagnostic or screening testing must have received an Emergency Use Authorization from the U.S. Food and Drug Administration (FDA) or be offered under the policies in FDA's [Policy for COVID-19 Tests](#) .

Tests used for SARS-CoV-2 **public health surveillance** on de-identified human specimens do not need to meet FDA and CLIA requirements for diagnostic and screening testing.

Reporting Diagnostic, Screening, and Public Health Surveillance Testing Results

Both **diagnostic** and **screening** testing results should be reported to the people whose specimens were tested and/or to their healthcare providers.

In addition, laboratories that perform diagnostic and screening testing must report test results (positive and negative) to the local, state, tribal, or territory health department in accordance with Public Law 116-136, § 18115(a), the Coronavirus Aid, Relief, and Economic Security (CARES) Act. The Department of Health and Human Services published guidance on [COVID-19 Pandemic Response, Laboratory Data Reporting: CARES Act Section 18115](#)   that specifies what data, in addition to test results, laboratories and testing sites should collect and electronically report.

Public health surveillance testing results cannot be reported directly to the people whose specimens have been tested and are not reported to their healthcare providers. Public health surveillance testing results (test results that are de-identified) can be reported in aggregate to local, state, tribal, or territory health departments upon request. Results from testing that is performed outside of a CLIA-certified facility or without an FDA-authorized test can only be reported to a health department if those results are used strictly for public health surveillance purposes, and not used for individual decision making.

Summary of Testing Strategies for SARS-CoV-2

Symptomatic
Diagnostic Yes
Screening No
Public Health Surveillance N/A

Unvaccinated or vaccinated with known or suspected exposure to SARS-CoV-2

Diagnostic
Yes

Screening
No

Public Health Surveillance
N/A

Unvaccinated and Asymptomatic without Known or Reported Suspected Exposure to SARS-CoV-2
Diagnostic No
Screening Yes
Public Health Surveillance N/A

Characterize Incidence and Prevalence in the Community

Diagnostic
N/A

Screening

N/A

Public Health Surveillance

Yes

Testing of Personally Identifiable Specimens

Diagnostic

Yes

Screening

Yes

Public Health Surveillance

No

Results may be Returned to Individuals

Diagnostic

Yes

Screening

Yes

Public Health Surveillance

No

Results Returned in Aggregate to Requesting Institution

Diagnostic

No

Screening

No

Public Health Surveillance

Yes

Results Reported to State Public Health Department

Diagnostic

Yes

Screening

Yes

Public Health Surveillance

If requested

Testing can be Performed in a CLIA-Certified Laboratory

Diagnostic

Yes

Screening

Yes

Public Health Surveillance

Yes

Testing can be Performed in a Non-CLIA-Certified Laboratory

Diagnostic

No

Screening

No

Public Health Surveillance

Yes

Test System Must be FDA Authorized or be Offered under the Policies in FDA's Guidance

Diagnostic

Yes

Screening

Yes

Public Health Surveillance

No

Resources

- [Overview of Testing for SARS-CoV-2 \(COVID-19\)](#)
- [FDA FAQs on Testing for SARS-CoV-2](#) 

Previous Updates



Updates from May 25, 2021

- Revised to align with [Interim Public Health Recommendations for Fully Vaccinated People](#)

Last Updated Dec. 28, 2021



COVID-19

Overview of Testing for SARS-CoV-2, the virus that causes COVID-19

Updated Jan. 21, 2022

CDC has updated [isolation and quarantine](#) recommendations for the public, and is revising the CDC website to reflect these changes. These recommendations do not apply to [healthcare personnel](#) and do not supersede state, local, tribal, or territorial laws, rules, and regulations.

Note: This document provides guidance on the different types of viral tests for SARS-CoV-2 available in the United States and their intended uses. It does not address issues regarding payment for or insurance coverage of such testing.

Summary of Recent Changes

Updates as of January 21, 2022



Revised to align with CDC's updated recommendations on [isolation and quarantine](#).

Revised to align with CDC recommendations for [people who are up to date with their vaccines](#).

[View Previous Updates](#)

Key Points

- People who have symptoms of COVID-19 or who have had known close contact to someone with COVID-19 should be tested for COVID-19.
- Point-of care serial screening testing can provide rapid results and is critical to identifying people with COVID-19 who do not have symptoms and slowing the spread of SARS-CoV-2. This is especially important when community risk or transmission levels are substantial or high.
- When selecting which SARS-CoV-2 test to use and interpreting results, healthcare providers, public health professionals, and those organizing and implementing testing should consider the context in which they are being used, including the prevalence of SARS-CoV-2 in the population being tested and the status (signs, symptoms, close contacts) of the person being tested.
- A person's vaccination status does not affect the results of their viral test for SARS-CoV-2.
- This guidance has been developed based on what is currently known about SARS-CoV-2 infection and COVID-19 and is subject to change as additional information becomes available.

A robust and responsive testing infrastructure is essential to the success of stopping the spread of SARS-CoV-2, the virus that causes COVID-19. This overview describes current information on the types of tests used to detect SARS-CoV-2 infection and their intended uses, including to diagnose infection, screening testing to reduce the virus's spread by people who do not have symptoms, and to monitor trends in infection. This guidance also includes considerations for:

- Health equity in testing
- Choosing a test
- Interpretating test results in vaccinated persons
- Testing in specific settings (e.g., K-12 schools, businesses, non-healthcare workplaces, correctional and detention facilities)
- Other considerations when deciding to test

This information is intended for use by healthcare providers, public health professionals, and those organizing and implementing testing in non-healthcare settings, such as schools, workplaces, and congregate housing. Information for the general public on SARS-CoV-2 testing is also [available](#).

Considerations When Testing

SARS-CoV-2 testing may be incorporated into a [comprehensive approach to reducing transmission](#) that also includes screening for [symptoms](#) and [contact tracing](#). When combined, these strategies can identify people infected with SARS-CoV-2 so that actions can be taken to slow and stop the spread of the virus.

People undergoing testing should [receive clear information](#) on

- The manufacturer and name of the test, the type of test, the purpose of the test, the performance specifications of the test, any limitations associated with the test, who will pay for the test, how the test will be performed, how and when people will receive test results, and;
- How to understand what the results mean, what actions need to happen after someone has negative or positive results, the difference between testing for workplace screening versus for medical diagnosis, who will receive the results, how the results may be used, and any consequences for declining to be tested.

Individuals tested are required to receive patient fact sheets as part of the test's [Emergency Use Authorization \(EUA\)](#).

Vaccination and SARS-CoV-2 Testing

If a person has received a COVID-19 vaccine, it does not affect the results of their SARS-CoV-2 viral tests (nucleic acid amplification tests [NAAT] or antigen). Because the Pfizer-BioNTech, Moderna, and Johnson & Johnson COVID-19 vaccines use the SARS-CoV-2 spike protein to generate an immune response, a positive serologic (antibody) test for spike protein IgM/IgG could indicate either previous infection or vaccination. Antibody testing is not currently recommended to assess for immunity to SARS-CoV-2 infection following COVID-19 vaccination or to assess the need for vaccination in an unvaccinated person. To evaluate for evidence of previous infection in a vaccinated individual, an antibody test specifically evaluating IgM/IgG to the nucleocapsid protein should be used (e.g., for public health surveillance or the diagnosis of Multisystem Inflammatory Syndrome in Children (MIS-C) or Multisystem Inflammatory Syndrome in Adults (MIS-A)). For guidance on quarantine and testing of [people who are up to date with their vaccines](#), please visit [COVID-19 Quarantine and Isolation | CDC](#).

Testing for SARS-CoV-2 Infection

Many types of tests are used to detect SARS-CoV-2,¹ and their performance characteristics vary.

- Some tests provide results rapidly (within minutes); others require time for processing.
- Some must be performed in a laboratory by trained personnel, some can be performed at the point of care, and others can be performed [at home](#) or anywhere.
- Some tests are very sensitive (i.e., few false-negative results or few missed detections of SARS-CoV-2); others are very specific (i.e., few false-positive results or few tests incorrectly identifying SARS-CoV-2 when the virus is not present); and

some are both sensitive and specific.

- Some tests can be performed frequently because they are less expensive and easier to use than other tests, and supplies are readily available.

When selecting which SARS-CoV-2 test to use healthcare providers, public health professionals, and those organizing and implementing testing should consider the context in which they are being used, including the prevalence of SARS-CoV-2 in the population being tested (See Table 1) and the status (signs, symptoms, close contacts) of the person being tested.

Test Types

Viral tests, including NAATs and antigen tests, are used as diagnostic tests to **detect current infection** with SARS-CoV-2 and to inform an individual's medical care. Viral tests can also be used as screening tests to reduce the transmission of SARS-CoV-2 by identifying infected persons who need to [isolate](#) from others. See FDA's list of [In Vitro Diagnostics Emergency Use Authorizations](#) for more information about the performance of specific authorized tests.

- [NAATs](#) are high-sensitivity, high-specificity tests for diagnosing SARS-CoV-2 infection. NAATs detect one or more viral ribonucleic acid (RNA) genes and indicate a current infection or a recent infection but, due to prolonged viral RNA detection, are not always direct evidence for the presence of virus capable of replicating or being transmitted to others. Most NAATs need to be processed in a laboratory, and time to results can vary (~1–3 days), but some NAATs are point-of-care tests with results available in about 15–45 minutes. Most NAATs produce qualitative results. NAATs can be performed on upper respiratory specimens, such as nasopharyngeal, nasal mid-turbinate, anterior nasal, or saliva.
- Antigen tests are immunoassays that detect the presence of a specific viral antigen. Antigen tests generally have similar specificity, but are less sensitive than most NAATs. Most are less expensive than NAATs and can be processed at the point of care with results available in minutes and thus can be used in screening programs to quickly identify those who are likely to be contagious. Because of the performance characteristics of antigen tests, it may be necessary to confirm some antigen test results (a negative test in persons with symptoms or a positive test in persons without symptoms) with a laboratory-based NAAT. However, based on the [authorization from FDA](#), some point-of-care [NAATs](#) that provide presumptive results cannot be used for confirmatory testing. Use of the [Antigen Testing Algorithm](#) is recommended to determine when confirmatory testing is needed.

Correct interpretation of results from both antigen tests and confirmatory NAATs, when indicated, is important.

Positive test results allow for identification and isolation of infected persons, as well as a case interview to identify and notify the case's close contact(s) of exposure and the need to quarantine.

Negative test results in persons with known SARS-CoV-2 exposure suggest no current evidence of infection. These results represent a snapshot of the time around specimen collection and could change if the same test was performed again in one or more days. For guidance on quarantine after a negative test, visit [COVID-19 Quarantine and Isolation | CDC](#). [In healthcare facilities with an outbreak of SARS-CoV-2](#), recommendations for viral testing of healthcare providers, residents, and patients (regardless of their vaccination status) remain unchanged.

Negative test results in persons who have no symptoms and no known exposure suggest no infection. All persons being tested, regardless of their results, should talk to their healthcare provider about [risk reduction behaviors](#) that help prevent the transmission of SARS-CoV-2 (e.g., wearing a [well-fitting mask](#), physical distancing, avoiding crowds and poorly ventilated spaces).

Antibody (or serology) tests are used to **detect previous infection** with SARS-CoV-2 and can aid in the diagnosis of [multisystem inflammatory syndrome in children \(MIS-C\)](#) [↗](#) and in [adults \(MIS-A\)](#)². CDC does not recommend using antibody testing to diagnose current infection. Depending on the time when someone was infected and the timing of the test, the test might not detect antibodies in someone with a current infection. In addition, it is not currently known whether a positive antibody test result indicates immunity against SARS-CoV-2; therefore, at this time, antibody tests should not be used to determine if an individual is immune against reinfection. Antibody testing is being used for public health surveillance and epidemiologic purposes. Because antibody tests can have different targets on the virus, specific tests might be needed to assess for antibodies originating from past infection versus those from vaccination. For more information about COVID-19 vaccines and antibody test results, refer to [Interim Clinical Considerations for Use of mRNA COVID-19 Vaccines Currently Authorized in the United States](#).

Overview of Testing Scenarios

Diagnostic testing is intended to identify current infection in individuals and is performed when a person has signs or symptoms consistent with COVID-19, or is asymptomatic, but has recent known or suspected close contact exposure to SARS-CoV-2.

Examples of diagnostic testing include:

- Testing persons with symptoms consistent with COVID-19, whether or not they are up to date on their vaccinations.
- Testing persons as a result of contact tracing efforts.
- Testing persons who indicate that they had close contact exposure with someone suspected or confirmed as having COVID-19.

Screening tests are recommended for those who have no symptoms and no known, suspected, or reported close contact exposure to SARS-CoV-2. Screening helps to identify unknown cases so that measures can be taken to prevent further transmission.

Examples of screening include:

- Testing employees in a workplace setting
- Testing students, faculty, and staff in a school or university setting
- Testing a person before or after travel
- Testing at home for someone who does not have symptoms associated with COVID-19 and no known exposures to someone with COVID-19

Public health surveillance is intended to monitor population-level burden of disease, or to characterize the incidence and prevalence of disease. Surveillance testing is primarily used to gain information at a population level, rather than an individual level, and generally involves testing of de-identified specimens. Surveillance testing results are not reported back to the individual. As such, surveillance testing cannot be used for an individual’s healthcare decision making or individual public health actions, such as isolation or quarantine.

An example of surveillance testing is [wastewater surveillance](#).

Choosing a Test

When choosing which test to use, it is important to understand the purpose of the testing (diagnostic or screening), performance of the test within the context of the level of community transmission, need for rapid results, and other considerations (See Table 1). For example, even a highly specific antigen test may have a poor positive predictive value (high number of false positives) when used in a community where prevalence of infection is low. As an additional example, use of a laboratory-based NAAT in a community with high transmission and increased test demand may result in diagnostic delays due to processing time and time to return results. Positive and negative predictive values of NAAT and antigen tests vary depending upon the pretest probability. Pretest probability considers both the prevalence of the level of community transmission as well as the clinical context of the individual being tested. Additional information is available on sensitivity, specificity, positive and negative predictive values for [antigen tests](#) and [antibody tests](#), and the relationship between [pretest probability and the likelihood of positive and negative predictive values](#)[458 KB, 1 Page]. Also see FDA’s letters to clinical laboratory staff and healthcare providers on the potential for [false-positive results with antigen tests](#) and the potential for [false-negative results with molecular tests if a genetic variant of SARS-CoV-2](#) is found in the part of the viral genome assessed by the test.

Table 1 summarizes some characteristics of NAATs and antigen tests to consider for a testing program. Given the risk of transmission of SARS-CoV-2 from asymptomatic and presymptomatic persons with SARS-CoV-2 infection, use of antigen tests in asymptomatic and presymptomatic persons can be considered. FDA has provided a list of [FAQs for healthcare providers who are using diagnostic tests in screening asymptomatic individuals](#), and the Centers for Medicare & Medicaid Services will [temporarily exercise enforcement discretion](#)  to enable the use of antigen tests that are not currently authorized for use in asymptomatic individuals for the duration of the COVID-19 public health emergency under the Clinical Laboratory

Improvement Amendments of 1988 (CLIA). Laboratories that perform screening or diagnostic testing for SARS-CoV-2 must have a CLIA certificate and meet regulatory requirements. Tests that have received an EUA from FDA for point-of-care (POC) use can be performed with a CLIA certificate of waiver.

Table 1. NAAT and Antigen Test Differences to Consider When Planning for Diagnostic or Screening Use

NAATs
Antigen Tests
Intended Use
Detect <i>current</i> infection
Detect <i>current</i> infection
Analyte Detected
Viral Ribonucleic Acid (RNA)
Viral Antigens
Specimen Type(s)
Nasal, Nasopharyngeal, Oropharyngeal, Sputum, Saliva
Nasal, Nasopharyngeal, Saliva
Sensitivity
Varies by test, but generally high for laboratory-based tests and moderate-to-high for POC tests
Varies depending on the course of infection, but generally moderate-to-high at times of peak viral load*
Specificity
High
High
Test Complexity

Varies by test

Relatively easy to use

Authorized for Use at the Point of Care

Most are not, some are

Most are, some are not

Turnaround Time

Most 1-3 days. Some could be rapid in 15 minutes

Ranges from 15 minutes to 30 minutes

Cost/Test^

Moderate (~\$75-\$100/test)

Low (~\$5-\$50/test)

Advantages

Most sensitive test method available

Short turnaround time for NAAT POC tests, but few available

Usually does not need to be repeated to confirm results

Short turnaround time (approximately 15 minutes)⁺

When performed at or near POC, allows for rapid identification of infected people, thus preventing further virus transmission in the community, workplace, etc.

Comparable performance to NAATs in symptomatic persons and/or if culturable virus present, when the person is presumed to be infectious

Disadvantages

Longer turnaround time for lab-based tests (1–3 days)

Higher cost per test

A positive NAAT diagnostic test should not be repeated within 90 days, because people may continue to have detectable RNA after risk of transmission has passed

May need [confirmatory testing](#)

Less sensitive (more false negative results) compared to NAATs, especially among asymptomatic people

*The decreased sensitivity of antigen tests might be offset if the POC antigen tests are repeated more frequently (i.e., serial testing at least weekly).

^ Costs for: [NAATs](#) 

+Refers to point-of-care antigen tests only.

Health Equity in SARS-CoV-2 Testing

CDC's [COVID-19 Response Health Equity Strategy](#) outlines a plan to reduce the disproportionate burden of COVID-19 among racial and ethnic minority populations and other population groups (e.g., essential and frontline workers, people living in rural or frontier areas) who have experienced a disproportionate burden of COVID-19. One component to move towards greater health equity and to stop transmission of SARS-CoV-2 is ensuring availability of resources, including access to testing for populations who have experienced longstanding, systemic health and social inequities. All population groups, including racial and ethnic minority groups, should have equal access to affordable, quality and timely SARS-CoV-2 testing – with fast turnaround time for results — for diagnosis and screening to reduce community transmission. Efforts should be made to address barriers that might overtly or inadvertently create inequalities in testing.

In addition, completeness of race and ethnicity data is an important factor in understanding the impact the virus has on racial and ethnic minority populations. The U.S. Department of Health and Human Services has [required laboratories and testing facilities to report](#)  race and ethnicity data to health departments, in addition to other data elements, for individuals tested for SARS-CoV-2 or diagnosed with COVID-19. Healthcare providers and public health professionals need to ask and record race and ethnicity for anyone receiving a reportable test result and ensure these data are reported with the person's test results in order to facilitate understanding the impact of COVID-19 on racial and ethnic minority populations.

In communities with a higher proportion of racial and ethnic minority populations and [other populations disproportionately affected](#) by COVID-19, health departments should ensure there is timely and equitable access to and availability of testing with fast result return, especially when the level of community transmission is substantial or high.

Some strategies to achieve this goal include:

- Use a [social vulnerability index](#) to assist in selecting testing sites.
- Assess the capacity of these sites to expand diagnostic and screening testing to meet the demand for impacted areas. This includes assessing the availability of free testing, wait times for testing and for results, and categories of available test (NAAT vs. antigen), as well as identifying and removing barriers to testing (e.g., alternatives to drive-through testing for a community where many do not have cars; availability of testing on evenings and weekends).
- Increase the availability of free testing sites in communities. Employers, community-based, and faith-based organizations can be important partners to increase the number of free, community-based testing sites. This expansion ensures that wait times both for testing and reporting of results are decreased, helping limit the spread of SARS-CoV-2.
- Increase public messaging about the importance of testing and communicate these messages in multiple languages and venues, particularly in communities at higher risk and disproportionately impacted by the virus.

Considerations for testing in different scenarios

Diagnostic testing

Testing persons with signs or symptoms consistent with COVID-19

Positive test results using a viral test (NAAT or antigen) in persons with signs or symptoms consistent with COVID-19 indicate that the person has COVID-19, independent of vaccination status of the person. A negative antigen test in persons with signs or symptoms of COVID-19 should be confirmed by NAAT, a more sensitive test. For more information, see the [Antigen Test Algorithm](#).

All persons (independent of vaccination status) with positive results should [isolate](#) at [home](#) or, if in a healthcare setting, be placed on [appropriate precautions](#) . Most people with COVID-19 have mild illness and can recover at home without medical care. For more information, see CDC's [COVID-19 quarantine and isolation](#) guidance.

NAATs have detected SARS-CoV-2 RNA in some people's respiratory specimens long after they have recovered from COVID-19 (>3 months). [Studies](#) have not found evidence that clinically recovered adults with persistence of viral RNA have transmitted SARS-CoV-2 to others. These findings support the recommendation for a symptom-based, rather than test-based, strategy for ending isolation of most people, so that individuals who are no longer infectious are not kept unnecessarily isolated and excluded from work or other responsibilities.

Some adults with severe illness may produce replication-competent virus beyond 10 days that may warrant extending duration of isolation and precautions. A test-based strategy may be considered in consultation with infectious disease experts for persons with severe illness or who are severely immunocompromised. For more information, including on retesting persons previously infected with SARS-CoV-2, visit [Duration of Isolation and Precautions for Adults with COVID-19](#).

Testing asymptomatic persons who have had recent known or suspected close contact exposure to SARS-CoV-2

Identifying [close contacts](#) (people who have been within 6 feet for a combined total of 15 minutes or more during a 24-hour period) of persons with COVID-19 can help reduce the spread of SARS-CoV-2 in communities, workplaces, and schools when these close contacts quarantine themselves. Viral testing is recommended for individuals who are close contacts of persons with COVID-19. Regardless of their vaccination status, people who have had a close contact exposure with someone known or suspected of having COVID-19 should be tested at least 5 days after the incident, if possible, or earlier if symptoms develop. [Most people](#) with a history of test-confirmed COVID-19 who remain symptom-free after recovery do not need to retest or quarantine if another exposure occurs within 90 days after their initial infection. For more information, see CDC's [COVID-19 quarantine and isolation guidance](#).

Negative test results using a viral test (NAAT or antigen) in asymptomatic persons with recent known or suspected close contact exposure suggest no current evidence of infection. These results represent a snapshot of the time around specimen collection and could change if tested again in one or more days. In instances of higher pretest probability, such as high incidence of infection in the community, or a person with household or continuous contact to a person with COVID-19, clinical judgement should determine if a positive antigen result for an asymptomatic person should be followed by a laboratory-based confirmatory NAAT. Results from NAATs are considered the definitive result when there is a discrepancy between the antigen and NAAT test. For more information, see the [Antigen Test Algorithm](#).

Because of the potential for asymptomatic and presymptomatic transmission, it is important that individuals [exposed to people with known or suspected COVID-19](#) be quarantined (if they are not up to date with their vaccines) or wear a [well-fitting mask](#) in public settings (if they are [up to date with their vaccines](#) or if they had confirmed COVID-19 within the past 90 days). Regardless of their vaccination status, persons with positive results should follow CDC's guidance for isolation. Those not up to date with their vaccines with negative results should remain in [quarantine](#) [↗](#) for 5 days unless other guidance is given by the local, tribal, or territorial public health authority.

Based on local circumstances and resources, [CDC has provided options to shorten quarantine](#), including the use of a test-based strategy. More information on the scientific foundation behind these recommendations is available in [Options to Reduce Quarantine for Contacts of Persons with SARS-CoV-2 Infection Using Symptom Monitoring and Diagnostic Testing](#).

Confidentiality of the individual with COVID-19 should be maintained when informing close contacts of their possible exposure to SARS-CoV-2. People are encouraged to work with public health departments investigating cases of COVID-19, including identification of close contacts.

Information to help public health departments and healthcare providers prepare for expanded viral testing in facilities after known or suspected SARS-CoV-2 exposure or when there are substantial or high levels of community transmission (Table 2) is available in CDC's [Performing Broad-Based Testing for SARS-CoV-2 in Congregate Settings](#).

Testing to determine resolution of infection

[Accumulating evidence supports ending isolation and precautions](#) [↗](#) for persons with COVID-19 using a symptom-based strategy. Adults with more severe illness or who are immunocompromised may remain infectious up to 20 days or longer after symptom onset, so a test-based strategy could be considered in consultation with infectious disease experts for these people. For all others, a test-based strategy is no longer recommended except to discontinue isolation or precautions earlier than would occur under the symptom-based strategy.

People with immunocompromising conditions or people who take immunosuppressive medications or therapies are at increased risk for severe COVID-19. Immunocompromised people ages 5 years and older should receive a primary COVID-19 vaccine series as soon as possible. In addition, [moderately or severely immunocompromised people](#) may not mount a protective immune response after initial vaccination, and their protection by primary vaccination may wane over time. Therefore, ACIP and CDC have made [age-specific recommendations](#) for an additional primary dose and a booster dose for this population including, but not limited to, people receiving chemotherapy for cancer, people with hematologic cancers such as chronic lymphocytic leukemia, people receiving stem cell or organ transplants, people receiving hemodialysis, and [people using certain medications](#) that might blunt the immune response to vaccination.

People who are immunocompromised should talk to a healthcare provider about the potential for reduced immune responses to COVID-19 vaccines and the need to continue to follow current prevention measures (including wearing a [well-fitting mask](#), [staying 6 feet apart from others](#) they don't live with, using self-tests, and avoiding crowds and poorly ventilated indoor spaces) to protect themselves against COVID-19 until advised otherwise by their healthcare provider. [Close contacts](#) of immunocompromised people should also stay up to date with their COVID-19 vaccinations to help protect these people.

Screening Testing

Testing asymptomatic persons without recent known or suspected exposure to SARS-CoV-2 for early identification, isolation, and disease prevention

Unvaccinated persons with asymptomatic or presymptomatic infection are frequent contributors to community SARS-CoV-2 transmission and occurrence of COVID-19 illness. Serial testing of unvaccinated persons, regardless of their signs or symptoms, is a key component to a layered approach to preventing the transmission of SARS-CoV-2. Screening allows early identification and isolation of persons who are asymptomatic, presymptomatic, or have only mild symptoms and who might be unknowingly transmitting virus. Screening testing may be most valuable in areas with substantial or high community transmission levels (Table 2), in areas with low vaccination coverage, and in certain settings (see examples below).

Use of POC tests, such as antigen tests, for screening can play an important role in testing as a prevention strategy due to the short turn-around time for results. Antigen tests are most sensitive in the early stages of infection when viral loads are high and have decreasing sensitivity as disease progresses and when transmission may be less likely. The decreased sensitivity of antigen tests might be offset if the POC antigen tests are repeated more frequently (i.e., serial testing at least weekly). Thus, when screening large numbers of persons (e.g., a well-defined cohort) without known or suspected exposure to SARS-CoV-2, test sensitivity may be less critical than whether the test can be performed more frequently and provide rapid results with immediate isolation of infected individuals.³ Outbreak prevention and control are increasingly thought to depend largely on the frequency of testing and the speed of reporting (an advantage of antigen tests) and is only marginally improved – in the context of serial tests — by the higher test sensitivity of NAATs. In screening settings where antigen tests are used on asymptomatic people, laboratory-based confirmatory NAAT testing may be needed for certain individuals who test positive. For interpretation of screening test results, please see the [Antigen Test Algorithms](#).

People without symptoms and without known exposure to COVID-19 do not need to quarantine while awaiting screening test results. If a person tests positive on a screening test and is referred for a confirmatory test, they should quarantine until they receive the results of their confirmatory test. For guidance on quarantine and testing of [people who are up to date with their vaccines](#), please visit [COVID-19 Quarantine and Isolation | CDC](#).

Screening testing as a prevention strategy

- Screening testing can improve detection of SARS-CoV-2.
 - Serial testing (within cohorts) with rapid isolation of infected individuals may facilitate re-opening of businesses, communities, and schools (in-person instruction in K-12 schools⁴) with less risk of a surge in local cases.
 - Frequent testing (1–2 times per week) combined with other risk reduction strategies, contributed to low case rates in university settings.⁵
- Frequency of testing could be informed by:
 - Current level of community transmission (Table 2), in addition to other known factors about the epidemiology of transmission in a particular cohort. If community transmission is substantial or high (Table 2), more frequent screening might be needed regardless of other indicators.

- Characteristics (e.g., size, proximity of people, duration of interaction) of the school, workplace, residential setting, or gathering.
- The [incubation period for COVID-19](#). Given that the incubation period can be up to 14 days, CDC recommends conducting screening testing at least weekly.
- Testing using a tiered approach, analogous to testing described in [high-density critical workplace](#) and [institutes of higher education](#) guidance, could be considered and might be particularly important for low incidence areas.
 - On some school campuses (e.g., institutes of higher learning), unvaccinated students may be tested upon arrival on campus or upon return from extended breaks.
- See additional guidance to facilitate implementation of [screening in congregate settings](#).

Examples of groups to prioritize for screening testing

These examples can guide development of local recommendations to prioritize **people not up to date with their vaccines** for screening testing, taking into account feasibility and costs. Initial sampling of subgroups for screening testing to evaluate the need for additional screening testing in a particular group may also be considered. In communities with substantial or high levels of community transmission (Table 2), health departments should ensure resources (trained staff and testing supplies) are available to provide expanded screening testing. **These examples are not listed in a priority order.**

Racial and ethnic minority groups and other [populations disproportionately affected](#) by COVID-19

Teachers and staff in [K-12 schools](#) and/or childcare settings

Students, faculty, and staff at [institutions of higher education](#) (including community colleges and technical schools)

Workers in [high-density worksites](#) or worksites with large numbers of [close contact](#) to co-workers or customers (restaurant workers, transportation workers, grocery store workers)

Government workers with public interactions as part of their duties (post office workers)

First responders (police, fire, emergency medical technician [EMT]) and [healthcare personnel](#)

Residents and staff in [congregate settings](#), such as shelters serving the [homeless](#) and [correctional and detention facilities](#) or residential settings ,such as [nursing homes](#) or those serving persons with disabilities; workplaces that provide congregate housing (fishing vessels, offshore platforms, farmworker housing or wildland firefighter camps); and military facilities (barracks)

Persons who recently traveled, either [domestically](#) or [internationally](#)

Persons who attended [large gatherings](#)

[Patients in healthcare settings](#)

Specific age groups (e.g., young adults) for whom increases in COVID19 have been documented early as incidence rises, especially in communities with substantial or high transmission (Table 2).

Table 2. Level of Community Transmission

Indicator	Low	Moderate	Substantial	High
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Indicator	Low	Moderate	Substantial	High
Cumulative number of new cases per 100,000 persons within the last 7 days*	<10	10-49	50-99	≥ 100
Percentage of NAATs that are positive during the last 7 days†	<5%	5%-7.9%	8%-9.9%	≥ 10.0%

Indicators should be calculated for counties or core based statistical areas, although in rural areas with low population density, multiple jurisdictions might need to be combined to make the indicators more useful for decision-making. The indicators listed can be found by county on CDC’s [COVID Data Tracker Website under “county view”](#).

* Number of new cases in the county (or other administrative level) in the last 7 days divided by the population in the county (or other administrative level) and multiplying by 100,000.

† Number of positive tests in the county (or other administrative level) during the last 7 days divided by the total number of tests resulted in the county (or other administrative level) during the last 7 days. [Calculating Severe Acute Respiratory Syndrome Coronavirus 2 \(SARS-CoV-2\) Laboratory Test Percent Positivity: CDC Methods and Considerations for Comparisons and Interpretation](#).

Public Health Surveillance Testing for SARS–CoV–2

Public health surveillance is the ongoing, systematic collection, analysis, and interpretation of health-related data essential to the planning, implementation, and evaluation of public health practice. See CDC’s [Introduction to Public Health Surveillance](#).

Public health surveillance testing is intended to monitor community- or population-level outbreaks of disease or to characterize the incidence and prevalence of disease. Surveillance testing is performed on de-identified specimens, and, thus, results are not linked to individual people. Public health surveillance testing results cannot be used for individual decision-making.

Public health surveillance testing may sample a certain percentage of a specific population to monitor for increasing or decreasing prevalence or to determine the population effect from community interventions, such as social distancing. An example of public health surveillance testing is when a state public health department develops a plan to randomly select and sample a percentage of all people in a city on a rolling basis to assess local infection rates and trends.

“Wastewater,” also referred to as “sewage,” includes water from household/building use (i.e., toilets, showers, sinks) that can contain human fecal waste, as well as water from non-household sources (e.g., rainwater and industrial use), can be tested for RNA from SARS-CoV-2. Data from wastewater testing are not meant to replace existing COVID-19 surveillance systems. Institutes of higher education with the resources to implement wastewater surveillance should develop a wastewater surveillance strategy in consultation with local public health authorities.

CDC is working with state, local, territorial, academic, and commercial partners to conduct surveillance to better understand COVID-19 in the United States and recently conducted a [multistate assessment of SARS-CoV-2 seroprevalence in blood donors](#).

- [Cases, Data, and Surveillance](#)
 - [COVID-19-Associated Hospitalization Surveillance Network \(COVID-NET\)](#)
 - [COVID-19 Serology Surveillance](#)
 - [National Wastewater Surveillance System \(NWSS\)](#)
 - [FAQ: COVID-19 Data & Surveillance](#)
- [Surveillance and Data Analytics](#)
- [CDC’s Diagnostic Multiple Assay for Flu and COVID-19 at Public Health Laboratories and Supplies](#)
- [Emerging SARS-CoV-2 Variants \(SARS-CoV-2 Strain Surveillance\)](#)

Setting-specific Testing Guidance

Testing in Healthcare Settings

[Nursing Homes](#)

[Acute Care Facilities](#)

[Infection Prevention and Control Recommendations for Healthcare Personnel](#)

Testing in Communities, Schools and Workplaces

[K-12 Schools](#)

[Institutes of Higher Education](#)

[Healthcare Personnel](#)

[Non-Healthcare Workplaces](#)

[High-Density Critical Workplaces](#)

[Homeless Shelters](#)

[Correctional and Detention Facilities](#)

Testing Information for the Public

[Testing Guidance for the Public](#)

[Testing and International Air Travel](#)

[Frequently Asked Questions: Testing](#)

Other Testing Resources

[Antigen Testing Algorithm](#)

[Interim Guidelines for COVID-19 Antibody Testing](#)

[Pooled Procedures for Testing](#)

[Laboratory Resources](#)

[Performing Facility-Wide Testing in Nursing Homes](#)

[Testing in K-12 Settings Playbook](#) [↗](#)

[Antigen Testing for Screening in Non-Healthcare Workplaces](#)

Previous Updates

Updates from Previous Content



As of October 22, 2021

- Based on evolving evidence, CDC recommends fully vaccinated people get tested 5-7 days after close contact with a person with suspected or confirmed COVID-19.

As of August 2, 2021

- Revised to align with CDC recommendations for fully vaccinated individuals

As of July 1, 2021

- Revised to align with CDC recommendations for fully vaccinated individuals

As of June 14, 2021

- Expansion on the description of categories of tests, choosing a test, and addition of intended uses of testing
- Addition of health equity considerations related to testing, including discussion on ensuring equitable testing access and availability

- Discussion on expanded availability to, and use of, screening tests to reduce asymptomatic spread
- Discussion on testing of vaccinated individuals and interpretation of test results
- Inclusion of links to setting-specific testing guidance

As of September 18, 2020

- Due to the significance of asymptomatic and pre-symptomatic transmission, this guidance further reinforces the need to test asymptomatic persons, including [close contacts](#) of a person with documented SARS-CoV-2 infection.

As of August 24, 2020

- Diagnostic testing categories have been edited to focus on testing considerations and actions to be taken by individuals undergoing testing

As of July 17, 2020

- Except for rare situations, a test-based strategy is no longer recommended to determine when an individual with a SARS-CoV-2 infection is no longer infectious (i.e., to discontinue Transmission-Based Precautions or home isolation)

As of July 2, 2020

- Added screening to possible testing types
- Removed examples – please refer to setting specific guidance

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Last Updated Jan. 21, 2022

FAQs on Testing for SARS-CoV-2

This section is primarily intended for SARS-CoV-2 test developers. For general public and consumer FAQs on COVID-19, please see [FDA's COVID-19 Frequently Asked Questions](https://www.fda.gov/emergency-preparedness-and-response/coronavirus-disease-2019-covid-19/covid-19-frequently-asked-questions) ([/web/20220113123605/https://www.fda.gov/emergency-preparedness-and-response/coronavirus-disease-2019-covid-19/covid-19-frequently-asked-questions](https://web/20220113123605/https://www.fda.gov/emergency-preparedness-and-response/coronavirus-disease-2019-covid-19/covid-19-frequently-asked-questions)).

This section provides answers to frequently asked questions relating to the development and performance of tests for SARS-CoV-2. These questions and answers provide additional clarity on existing policies and do not introduce any new policies or modify any existing policies.

This section includes questions and answers regarding the policies described in the *Policy for Coronavirus Disease-2019 Tests During the Public Health Emergency (Revised)* ([/web/20220113123605/https://www.fda.gov/regulatory-information/search-fda-guidance-documents/policy-coronavirus-disease-2019-tests-during-public-health-emergency-revised](https://web/20220113123605/https://www.fda.gov/regulatory-information/search-fda-guidance-documents/policy-coronavirus-disease-2019-tests-during-public-health-emergency-revised)), originally issued as *Policy for Diagnostic Tests in Laboratories Certified to Perform High Complexity Testing under CLIA prior to Emergency Use Authorization for Coronavirus Disease-2019 during the Public Health Emergency* on February 29, 2020 and updated on March 16, 2020, May 4, 2020, May 11, 2020, and November 15, 2021. In this section, this guidance is referred to as the *Policy for Coronavirus Disease-2019 Tests*.

Tests being offered prior to or without an EUA as described in a policy in a previous version or the current version of the *Policy for Coronavirus Disease-2019 Tests* ([/web/20220113123605/https://www.fda.gov/regulatory-information/search-fda-guidance-documents/policy-coronavirus-disease-2019-tests-during-public-health-emergency-revised](https://web/20220113123605/https://www.fda.gov/regulatory-information/search-fda-guidance-documents/policy-coronavirus-disease-2019-tests-during-public-health-emergency-revised)) have not been reviewed or authorized by the FDA. As stated in the current and previous versions of the *Policy for Coronavirus Disease-2019 Tests* ([/web/20220113123605/https://www.fda.gov/regulatory-information/search-fda-guidance-documents/policy-coronavirus-disease-2019-tests-during-public-health-emergency-revised](https://web/20220113123605/https://www.fda.gov/regulatory-information/search-fda-guidance-documents/policy-coronavirus-disease-2019-tests-during-public-health-emergency-revised)), all such tests should be validated by the developer prior to being offered for clinical use.

Note: Throughout this section and the *Policy for Coronavirus Disease-2019 Tests* ([/web/20220113123605/https://www.fda.gov/regulatory-information/search-fda-guidance-documents/policy-coronavirus-disease-2019-tests-during-public-health-emergency-revised](https://web/20220113123605/https://www.fda.gov/regulatory-information/search-fda-guidance-documents/policy-coronavirus-disease-2019-tests-during-public-health-emergency-revised)), references to laboratories that are "certified to perform high complexity testing under CLIA" or to "high-complexity CLIA-certified laboratories" are referring to CLIA-certified laboratories that meet the regulatory requirements to perform high-complexity testing.

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(/web/20220113123605/https://www.fda.gov/medical-devices/coronavirus-covid-19-and-medical-devices/notifications-and-emergency-use-authorizations-faqs-testing-sars-cov-2)
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- [Testing Supplies](/web/20220113123605/https://www.fda.gov/medical-devices/coronavirus-covid-19-and-medical-devices/covid-19-testing-supplies-faqs-testing-sars-cov-2) (/web/20220113123605/https://www.fda.gov/medical-devices/coronavirus-covid-19-and-medical-devices/covid-19-testing-supplies-faqs-testing-sars-cov-2)
- [COVID-19 Related Test Data and Reporting](/web/20220113123605/https://www.fda.gov/medical-devices/coronavirus-covid-19-and-medical-devices/covid-19-related-test-data-and-reporting-faqs-testing-sars-cov-2)
(/web/20220113123605/https://www.fda.gov/medical-devices/coronavirus-covid-19-and-medical-devices/covid-19-related-test-data-and-reporting-faqs-testing-sars-cov-2)

COVID-19 Resources for Test Developers

- **[Emergency Use Authorizations for Medical Devices](/web/20220113123605/https://www.fda.gov/medical-devices/emergency-use-authorizations-medical-devices/coronavirus-disease-2019-covid-19-emergency-use-authorizations-medical-devices)**
(/web/20220113123605/https://www.fda.gov/medical-devices/emergency-use-authorizations-medical-devices/coronavirus-disease-2019-covid-19-emergency-use-authorizations-medical-devices): Includes EUAs for [In Vitro Diagnostics](/web/20220113123605/https://www.fda.gov/medical-devices/coronavirus-disease-2019-covid-19-emergency-use-authorizations-medical-devices/in-vitro-diagnostics-euas) (/web/20220113123605/https://www.fda.gov/medical-devices/coronavirus-disease-2019-covid-19-emergency-use-authorizations-medical-devices/in-vitro-diagnostics-euas) (e.g., molecular, antigen, and serology tests)
- **[Coronavirus \(COVID-19\) and Medical Devices: In Vitro Diagnostic Tests](https://web.archive.org/web/20220113123605/https://www.fda.gov/medical-devices/emergency-situations-medical-devices/coronavirus-covid-19-and-medical-devices#ivd)**
(https://web.archive.org/web/20220113123605/https://www.fda.gov/medical-devices/emergency-situations-medical-devices/coronavirus-covid-19-and-medical-devices#ivd)
- **[SARS-CoV-2 Viral Mutations: Impact on COVID-19 Tests](/web/20220113123605/https://www.fda.gov/medical-devices/coronavirus-covid-19-and-medical-devices/sars-cov-2-viral-mutations-impact-covid-19-tests)**
(/web/20220113123605/https://www.fda.gov/medical-devices/coronavirus-covid-19-and-medical-devices/sars-cov-2-viral-mutations-impact-covid-19-tests)
- **[FAQs on Viral Transport Media During COVID-19](/web/20220113123605/https://www.fda.gov/medical-devices/coronavirus-covid-19-and-medical-devices/faqs-viral-transport-media-during-covid-19)**
(/web/20220113123605/https://www.fda.gov/medical-devices/coronavirus-covid-19-and-medical-devices/faqs-viral-transport-media-during-covid-19): Includes questions and answers regarding the

policies outlined in the *Enforcement Policy for Viral Transport Media During the Coronavirus Disease 2019 (COVID-19) Public Health Emergency* ([/web/20220113123605/https://www.fda.gov/regulatory-information/search-fda-guidance-documents/enforcement-policy-viral-transport-media-during-coronavirus-disease-2019-covid-19-public-health](https://www.fda.gov/regulatory-information/search-fda-guidance-documents/enforcement-policy-viral-transport-media-during-coronavirus-disease-2019-covid-19-public-health)).

- **EUA Authorized Serology Test Performance** ([/web/20220113123605/https://www.fda.gov/medical-devices/coronavirus-disease-2019-covid-19-emergency-use-authorizations-medical-devices/eua-authorized-serology-test-performance](https://www.fda.gov/medical-devices/coronavirus-disease-2019-covid-19-emergency-use-authorizations-medical-devices/eua-authorized-serology-test-performance)).
- **Independent Evaluations of COVID-19 Serological Tests**
(<https://web.archive.org/web/20220113123605/https://open.fda.gov/apis/device/covid19serology/>).
- **SARS-CoV-2 Reference Panel Comparative Data** ([/web/20220113123605/https://www.fda.gov/medical-devices/coronavirus-covid-19-and-medical-devices/sars-cov-2-reference-panel-comparative-data](https://www.fda.gov/medical-devices/coronavirus-covid-19-and-medical-devices/sars-cov-2-reference-panel-comparative-data)).
- **Pooled Sample Testing and Screening Testing for COVID-19**
([/web/20220113123605/https://www.fda.gov/medical-devices/coronavirus-covid-19-and-medical-devices/pooled-sample-testing-and-screening-testing-covid-19](https://www.fda.gov/medical-devices/coronavirus-covid-19-and-medical-devices/pooled-sample-testing-and-screening-testing-covid-19)).
- **Virtual Town Hall Series - Coronavirus (COVID-19) Test Development and Validation**
([/web/20220113123605/https://www.fda.gov/medical-devices/workshops-conferences-medical-devices/virtual-town-hall-series-coronavirus-covid-19-test-development-and-validation-01122022-01122022](https://www.fda.gov/medical-devices/workshops-conferences-medical-devices/virtual-town-hall-series-coronavirus-covid-19-test-development-and-validation-01122022-01122022)).

Reporting Problems to the FDA

The sale of fraudulent COVID-19 products is a threat to the public health. Consumers and health care professionals can help by reporting suspected fraud to the FDA's Health Fraud Program ([/web/20220113123605/https://www.fda.gov/safety/report-problem-fda/reporting-unlawful-sales-medical-products-internet](https://www.fda.gov/safety/report-problem-fda/reporting-unlawful-sales-medical-products-internet)) or the Office of Criminal Investigations ([https://web.archive.org/web/20220113123605/https://www.accessdata.fda.gov/scripts/email/oc/oci/contact.cfm](https://www.accessdata.fda.gov/scripts/email/oc/oci/contact.cfm)). You can also email FDA-COVID-19-Fraudulent-Products@fda.hhs.gov (<https://web.archive.org/web/20220113123605/mailto:FDA-COVID-19-Fraudulent-Products@fda.hhs.gov>). [https://web.archive.org/web/20220113123605mp_/http://www.fda.gov/about-fda/website-policies/website-disclaimer](https://www.fda.gov/about-fda/website-policies/website-disclaimer)). If you are a health professional, consumer or patient and think you had a problem with your diagnostic test, the FDA encourages you to report the problem through the MedWatch Voluntary Reporting Form ([https://web.archive.org/web/20220113123605/https://www.accessdata.fda.gov/scripts/medwatch/index.cfm?action=reporting.home](https://www.accessdata.fda.gov/scripts/medwatch/index.cfm?action=reporting.home)). Health care personnel employed by facilities performing COVID-19 testing should follow the reporting procedures established by their facilities.



COVID-19

Overview of Testing for SARS-CoV-2 (COVID-19)

Updated Oct. 21, 2020

Note: This document is intended to provide guidance on the appropriate use of testing for SARS-CoV-2 in light of additional testing capacity throughout the country and does not address decisions regarding payment for or insurance coverage of such testing.

Summary of Recent Changes

Updates as of October 21, 2020



As of October 21, 2020

- Added links to the updated close contact definition.
- Updated language to align with updated definition and to align with sitewide changes

[View Previous Updates](#)

This document provides a summary of considerations and current Centers for Disease Control and Prevention (CDC) recommendations regarding SARS-CoV-2 testing strategy. The CDC recommendations for SARS-CoV-2 testing have been developed based on what is currently known about COVID-19 and are subject to change as additional information becomes available.

Viral Testing

Authorized assays for viral testing include those that detect SARS-CoV-2 nucleic acid or antigen. [Viral \(nucleic acid or antigen\) tests](#) check samples from the respiratory system (such as nasal or oral swabs or saliva) to determine whether an infection with SARS-CoV-2, the virus that causes COVID-19, is present. Viral tests are recommended to diagnose acute infection of both symptomatic and asymptomatic individuals, to guide contact tracing, treatment options, and isolation requirements. Some tests are point-of-care tests, meaning results may be available at the testing site in less than an hour. Other tests must be sent to a laboratory, a process that may take at least 1-2 days.

For more information on testing for SARS-CoV-2 see the [Interim Guidelines for Collecting, Handling, and Testing Clinical Specimens](#) and [Biosafety FAQs](#) for handling and processing specimens from possible cases.

Antibody Testing

FDA has not authorized using antibody tests to diagnose SARS-CoV-2 infection, and CDC does not currently recommend [using antibody testing](#) as the sole basis for diagnosis of acute infection. In certain situations, serologic assays may be used to support clinical assessment of persons who present late in their illnesses when used in conjunction with viral detection tests. In addition, if a person is suspected to have a post-infectious syndrome caused by SARS-CoV-2 infection (e.g., Multisystem Inflammatory Syndrome in Children; MIS-C), serologic assays may be used.

[Serologic assays](#) for SARS-CoV-2 infection, are an important tool for surveillance and epidemiologic studies, such as understanding the transmission dynamic of the virus in the general population. Unlike direct viral detection methods, such as nucleic acid amplification or antigen detection tests that can detect acutely infected persons, antibody tests help determine whether the individual being tested was previously infected—even if that person never showed symptoms.

CDC Mitigation Protocols

CDC recommends the following [measures to mitigate the spread of the virus](#) and to protect [people at increased risk of severe illness](#): social distancing, wearing a mask when social distancing is not possible, avoiding crowds, avoiding indoor crowded spaces, and washing or sanitizing hands frequently.

Considerations for SARS-CoV-2 Diagnostic (Molecular or Antigen) Testing

- If you have symptoms of COVID-19:
 - If your symptoms are mild:
 - Your healthcare provider (physician, nurse practitioner, pharmacist, etc.) may advise a SARS-CoV-2 test.
 - If you test positive for SARS-CoV-2 infection or do not get tested, you should [self-isolate](#) for at least 10 days after symptom onset and resolution of fever for at least 24 hours, without the use of fever-reducing medications, and with improvement of other symptoms.
 - If you live with a [person at increased risk of severe illness](#) (for example an elderly relative or other individuals with underlying conditions), take special precautions in the home to protect that individual according to CDC [guidelines](#).
 - If your symptoms worsen while you are isolating, or become moderate/severe, such as shortness of breath or severe fatigue, contact your healthcare provider immediately or seek emergency care.
 - If you test positive, you do not need to repeat a test [for at least 3 months](#).
 - You do not need a follow-up negative test to return to work or school, as long as
 - You did not require hospitalization, AND
 - It has been at least at least 10 days after symptom onset and resolution of fever for at least 24 hours, without the use of fever-reducing medications, and with improvement of other symptoms.
- If you have been in [close contact](#) (less than 6 feet for a total of 15 minutes or more) with documented SARS-CoV-2 infection and do not have symptoms.
 - You need a test. Please consult with your healthcare provider or public health official. Testing is recommended for all [close contacts](#) of persons with SARS-CoV-2 infection. Because of the potential for asymptomatic and pre-symptomatic transmission, it is important that contacts of individuals with SARS-CoV-2 infection be quickly identified and tested. Pending test results, you should [self-quarantine/isolate](#) at home and stay separated from household members to the extent possible and use a separate bedroom and bathroom, if available.
 - A single negative test does not mean you will remain negative at any time point after that test.
 - Even if you have a negative test, you should still self-isolate for 14 days.
 - If you cannot self-isolate, or you are a critical infrastructure worker that must work, wear a mask, physically distance, avoid crowds and indoor crowded places, wash your hands frequently, and monitor yourself for symptoms.
 - If you live with a [person at increased risk of severe illness](#) (for example an elderly person or other individuals with underlying medical conditions), take special precautions in the home to protect that individual according to CDC [guidelines](#).
 - Healthcare providers in [close contact](#) of a person with documented SARS-CoV-2 infection while using [recommended personal protective equipment](#), do not need to be tested
- If you do not have COVID-19 symptoms and have not been in [close contact](#) with someone known to have SARS-CoV-2 infection (meaning being within 6 feet of an infected person for at least 15 minutes).
 - You do not need a test unless recommended or required by your healthcare provider or public health official.
 - If you are tested, you should [self-quarantine](#) at home until your test results are known, and then adhere to your healthcare provider's advice.

– A negative test does not mean you will remain negative at any time point after that test.

- **If you are in a high SARS-CoV-2 transmission zone and attended a public or private gathering of more than 10 people (without universal mask wearing and/or physical distancing).**
 - Your healthcare provider or public health official may advise a SARS-CoV-2 test.
 - If you are tested, you should self-isolate at home until your test results are known, and then adhere to your healthcare provider’s advice.
 - A negative test does not mean you will remain negative at any time point after that test.
 - Even if you have a negative test, you should wear a mask, physically distance, avoid crowds and indoor crowded places, wash your hands frequently, and monitor yourself for symptoms.
 - Take special precautions in the home to protect any person(s) [with increased risk of severe illness](#) according to CDC [guidelines](#).
- **If you [work in a nursing home](#),**
 - You will need testing as part of a nursing home’s plan to open or reopen, if not previously tested.
 - You need to be tested if you are symptomatic. You must self-isolate and not go to work until your test results are known. If you test positive, you should [self-isolate](#) for at least 10 days after symptom onset and resolution of fever for at least 24 hours, without the use of fever-reducing medications, and with improvement of other symptoms.
 - You will need testing if there is an outbreak in your facility, and repeat testing at regular intervals if your initial test result was negative until the outbreak is over.
 - You will need serial testing based on your county percent test positivity rate.
 - Results of testing will be used to inform infection control interventions, such as resident placement decisions and work exclusions.
 - Follow any additional guidance from public health officials and the Centers for Medicare and Medicaid Services (CMS).
- **If you [live in or receive care in a nursing home](#),**
 - You will need testing as part of a nursing home’s plan to open or reopen, if not previously tested.
 - You need to be tested if you are symptomatic. You must be isolated until your test results are known. If you test positive, you should be isolated for at least 10 days after symptom onset and resolution of fever for at least 24 hours, without the use of fever-reducing medications, and with improvement of other symptoms.
 - You will also need to be tested if there is an outbreak in your facility. This will need to be repeated at regular intervals if your initial test result was negative, until the outbreak is over.
 - You may also need to be tested if you leave the facility on a regular basis (e.g. for dialysis or frequent medical/other appointments).
 - Results of testing will be used to inform infection control interventions, such as resident and patient placement decisions.
 - Follow any additional guidance from public health officials and the Centers for Medicare and Medicaid Services (CMS).
- **If you are a critical infrastructure worker, healthcare worker, or first responder.**
 - You may need to get tested according to your employer’s guidelines.
 - Even if you have a negative test, you should wear a mask when you cannot physically distance during your workday, avoid crowds and indoor crowded places, wash your hands frequently, and monitor yourself for symptoms.
- **For public health reasons, your public health official(s) or healthcare provider may advise specific people, or groups of people, to be tested. You should follow this advice. It is important to realize that you can be infected and spread the virus but feel totally well and have no symptoms.**
 - Your healthcare provider or public health official may recommend that you are tested before being admitted to the hospital or before a procedure (e.g., pregnant people admitted for labor and delivery, surgery). This testing is to protect healthcare personnel and other patients.
 - In areas where there are a small number of new cases and limited spread, your public health department may request a small number of asymptomatic “healthy people” to be tested.
 - If there is significant spread of the virus in your community, your public health department may request significant numbers of asymptomatic “healthy people” to be tested in order to help stop the spread of the virus.

- Certain settings can experience rapid spread of SARS-CoV-2. This is particularly true for settings with [people at increased risk of severe illness](#) in close quarters for extended periods of time.
 - Approaches for early identification of infected individuals include, initial testing of everyone in the setting, periodic (e.g., weekly) testing of everyone in the setting, and testing of new or returning entrants into the setting.

Previous Updates

Updates from Previous Content



As of September 18, 2020

- Due to the significance of asymptomatic and pre-symptomatic transmission, this guidance further reinforces the need to test asymptomatic persons, including [close contacts](#) of a person with documented SARS-CoV-2 infection.

As of August 24, 2020

- Diagnostic testing categories have been edited to focus on testing considerations and actions to be taken by individuals undergoing testing

As of July 17, 2020

- Except for rare situations, a test-based strategy is no longer recommended to determine when an individual with a SARS-CoV-2 infection is no longer infectious (i.e., to discontinue Transmission-Based Precautions or home isolation)

As of July 2, 2020

- Added screening to possible testing types
- Removed examples – please refer to setting specific guidance

Last Updated Oct. 21, 2020



COVID-19

 We have the tools to fight Omicron: vaccines and booster, masks, and testing.

Test for Current Infection

Updated Jan. 5, 2022

CDC has updated [isolation and quarantine](#) recommendations for the public, and is revising the CDC website to reflect these changes. These recommendations do not apply to [healthcare personnel](#) and do not supersede state, local, tribal, or territorial laws, rules, and regulations.

Viral tests are used to look for current infection

A viral test checks specimens from your nose or your mouth to find out if you are currently infected with the virus that causes COVID-19. Viral tests can be performed in a laboratory, at a testing site, or [at home or anywhere else](#). Two types of viral tests are used: [nucleic acid amplification tests \(NAATs\)](#) and [antigen tests](#).



Free At-Home COVID-19 Tests: [Order 4 free tests now so you have them when you need them.](#) [↗](#)

Who should get tested

The following people should get tested for COVID-19:

- People who have [symptoms of COVID-19](#).
- People who have come into [close contact](#) with someone with COVID-19 should be tested to check for infection at least 5 days after they last had close contact with someone with COVID-19. The date of the last close contact is considered day 0.
- People who are not [up to date with their COVID-19 vaccines](#) who are prioritized for expanded [community screening](#) for COVID-19.
- People regardless of vaccination status who have been asked or referred to get testing by their school, workplace, healthcare provider, [state](#), [tribal](#), [local](#) [↗](#) or [territorial health department](#).

Who does not need to be tested

The following people who have been exposed to someone with COVID-19 do **not** need to get tested if they do not have COVID-19 symptoms:

- People who have [tested positive for COVID-19 within the past 3 months and recovered](#), as long as they do not develop new symptoms, do not need to get tested.

COVID-19 viral testing tool

The COVID-19 Viral Testing Tool is an interactive web tool designed to help both healthcare providers and individuals understand COVID-19 testing options. This tool helps healthcare providers quickly access the most relevant, actionable information to determine what type(s) of COVID-19 testing they should recommend to patients. The tool helps individuals determine what type of test they should seek. After test results are in, the tool can help interpret test results and guide next steps.

The online, mobile-friendly tool asks a series of questions, and provides recommended actions and resources based on the user's responses.

How to get a viral test

- Visit your [state](#), [tribal](#), [local](#) [link icon](#), or [territorial](#) health department's website to look for the latest local information on testing.
- Visit your healthcare or public health department clinic provider to get a self-collection kit or self-test.
- You and your healthcare provider might consider either [self-collection kit](#) or a [self-test](#) if you have symptoms of COVID-19 and can't get tested by a healthcare provider.

Positive viral test

If you test positive for the virus that causes COVID-19, take the following steps to protect others regardless of your COVID-19 vaccination status:

- [Isolate](#) for at least 5 days. You can end isolation after 5 full days if you are fever-free for 24 hours without the use of fever-reducing medication and your other symptoms have improved (Loss of taste and smell may persist for weeks or months after recovery and need not delay the end of isolation). Day 0 is your first day of symptoms. You can also use a [test-based strategy](#) to end isolation.
- If you test positive for COVID-19 and never develop [symptoms](#), isolate for at least 5 days and wear a [well-fitting mask](#) around others at home and in public for an additional 5 days. Day 0 is the day the sample was collected for a positive test result.
- Wear a [well-fitting mask](#) around others at home and in public for 5 additional days after the end of your 5-day isolation period. Take additional precautions on days 6-10, such as avoiding travel and situations where you will come in contact with individuals at [high risk for severe disease](#). If you are unable to wear a mask when around others, you should continue to isolate for 10 days.
- Follow CDC's [recommendations for isolation](#).
- Contact your healthcare provider as soon as possible if you are more likely to get very sick because of being an older adult or having [underlying medical conditions](#) or if your symptoms get worse.



Talk to your healthcare provider or local health department to find out how long to isolate if you:

- Are severely ill with COVID-19 or have a weakened immune system;
- Had a positive test result followed by a negative result; or
- Test positive for many weeks after the initial result.

Negative viral test

If you test negative for the virus that causes COVID-19, the virus was not detected.

If you have symptoms of COVID-19:

- You may have received a false negative test result and still might have COVID-19. You should [isolate](#) away from others.
- Contact your healthcare provider about your symptoms, especially if they worsen, about follow-up testing, and how long to isolate.

If you do not have symptoms of COVID-19, and you were a [close contact](#) to someone with COVID-19:

- You are likely not infected, but you still may get sick.
- [Quarantine](#) if you are not vaccinated or have not completed a [primary vaccine series](#), or if you have not received all [recommended](#) booster shots.
 - If symptoms develop during home quarantine, contact your healthcare provider about follow-up testing; and
 - [Isolate at home](#) separated away from others.

If you do not have symptoms of COVID-19 and do not have a known exposure to a person with COVID-19:

- You do not need to quarantine.

Take steps to protect yourself

Whether you test positive or negative for COVID-19, you should take preventive measures to [protect yourself and others](#).

Last Updated Jan. 5, 2022



COVID-19

We have the tools to
Fight Omicron



Vaccines & Booster



Masks



Testing

Self-Testing

Updated Dec. 29, 2021

CDC has updated [isolation and quarantine](#) recommendations for the public, and is revising the CDC website to reflect these changes. These recommendations do not apply to [healthcare personnel](#) and do not supersede state, local, tribal, or territorial laws, rules, and regulations.



Free At-Home COVID-19 Tests: [Order 4 free tests now so you have them when you need them.](#) [↗](#)

Summary of Recent Changes

Updates as of December 6, 2021



- Added information on when and why to use a self-test. Self-testing is one of several risk-reduction strategies to reduce the chances of COVID-19 transmission. They may be used regardless of vaccination or symptom status.

[Previous updates](#)

Key Points

- COVID-19 self-tests (also referred to as home tests or over-the-counter (OTC) tests) are one of many risk-reduction measures, along with [vaccination](#), [masking](#), and physical distancing, that [protect you and others](#) by reducing the chances of spreading SARS-CoV-2, the virus that causes COVID-19.
- Self-tests can be taken at home or anywhere, are easy to use, and produce rapid results.
- You can use self-tests, regardless of vaccination status, or whether or not you have symptoms.
- Follow all of the manufacturer's instructions for performing the test.
- [If you test positive](#), you should [isolate](#) and inform your healthcare provider, as well as any [close contacts](#).
- Consider using a self-test before joining indoor gatherings with others who are not in your household.
- A positive self-test result means that the test detected the virus, and you are very likely to have an infection and should stay home or isolate for 10 days, wear a mask if you could have contact with others, and avoid indoor gatherings to reduce the risk of spreading disease to someone else.

- A negative self-test result means that the test did not detect the virus and you may not have an infection, but it does not rule out infection. Repeating the test within a few days, with at least 24 hours between tests, will increase the confidence that you are not infected.

Overview

Note: If your COVID-19 test is positive, tell a healthcare provider about your positive result and stay in contact with them during your illness. To avoid spreading the virus to others, follow CDC's [guidance for isolation](#). Tell your [close contacts](#) that they have been exposed to the virus that causes COVID-19.



Testing is critically important to help reduce the spread of the virus that causes COVID-19. If you have symptoms or had a known exposure to someone with suspected or confirmed COVID-19, you should be tested, regardless of your vaccination status. You may also consider self-testing to screen for COVID-19 immediately before an activity to see if you are positive for COVID-19. You should [isolate](#) if you test positive.

If you need to be tested for COVID-19, consider using a **self-test** that can be performed at home or anywhere else. Sometimes a self-test is also called a “home test”, an “at-home test”, or an “over-the-counter (OTC) test.” Visit [FDA's website](#) [↗](#) for a list of authorized tests. Some self-tests may have age limitations.

Self-testing offers fast results. Self-tests are one of several options for testing for the virus that causes COVID-19 and may be more convenient than laboratory-based tests and point-of-care tests. Consider keeping self-tests at home or where you may need them.

Self-tests can also be used as one of many risk-reduction measures, along with vaccination, masking, and physical distancing, that [protect you and others](#) by reducing the chance of viral transmission. You can self-test, even if you don't have symptoms or are fully vaccinated, in order to make decisions that will help prevent spreading COVID-19 to others.

Self-tests can be purchased online or in pharmacies and retail stores. They are also available for free through some [local health departments](#) [↗](#) or Federally Qualified Health Centers (FQHC). As of November 2021, self-tests are available to detect [current infection](#). Self-tests are not available to detect antibodies to the virus that causes COVID-19.

When to Consider Self-Testing

Self-tests may be used if you have COVID-19 symptoms or have been exposed or potentially exposed to an individual with COVID-19.

Even if you don't have symptoms and have not been exposed to an individual with COVID-19, using a self-test before gathering indoors with others can give you information about the risk of spreading the virus that causes COVID-19. This is especially important before gathering with [unvaccinated children](#), [older individuals](#), those who are [immunocompromised](#), or [individuals at risk of severe disease](#).

A positive test result indicates that you likely have a current infection, and you should [isolate](#) and inform [close contacts](#).

A negative test result indicates that you may not be infected and may be at low risk of spreading disease to others, though it does not rule out an infection. Repeating the test will increase the confidence that you are not infected. Performing serial tests, meaning two or more tests over several days with at least 24 hours between tests—with one test as close as possible to the event you will attend—improves the reliability of testing and reduces your risk of transmitting disease to others even further. Some self-tests require this type of repeat testing in the manufacturer’s instructions.

How to Use a Self-Test

Read the complete manufacturer’s instructions for use before using the test. Talk to a healthcare provider if you have questions about the test or your results. Refer to CDC’s videos on [How to Use a Self-Test](#) and [How to Interpret Self-Test Results](#).

Prepare to Collect a Specimen

- Wash your hands with soap and water for at least 20 seconds.
- Open the box and follow the instructions included with the self-test to collect your own nasal or saliva specimen.
- If you don’t collect the specimens as directed, your test results may be incorrect.

Perform the Test

Follow the instructions exactly and perform the steps in the order that they are listed. The manufacturer may also provide other resources, such as quick reference guides or instructional videos, to help you perform the test correctly.

Most self-tests require the collection of a **nasal specimen** (see the [Additional Print Resources](#) section below). A few self-tests require a **saliva specimen**.

Once collected, use the specimen as described in the instructions to complete the self-test.

 **Watch Video:** [How to use a self test](#) [00:02:45]

Tips

- Store all test components according to the manufacturer’s instructions until ready for use.
- Check the expiration date. Do not use expired tests or test components that are damaged or appear discolored based on the manufacturer’s instructions.
- [Clean](#) the countertop, table, or other surfaces where you will do the test.
- Do not open test devices or other test components until you are ready to start the testing process.
- Have a timer ready because you may need to time several of the test steps.
- Read test results only within the amount of time specified in the manufacturer’s instructions. A result read before or after the specified timeframe may be incorrect.
- Don’t reuse test devices or other components.

After you have the results, discard the specimen collection swab or tube and test in the trash, [clean all surfaces](#) that the specimen may have touched, and wash your hands.

If Your Test Result Is Positive

You should stay home or isolate for 10 days and wear a mask if others could have contact with you. Also, tell a healthcare provider about your positive test result and stay in contact with them. If your illness becomes severe, seek medical attention. If you have an [emergency warning sign](#) (including trouble breathing), seek emergency medical care immediately. To avoid spreading the virus to others, follow [CDC recommendations](#).

Tell your [close contacts](#) that they may have been exposed to the virus that causes COVID-19. A person with COVID-19 can begin spreading it starting 48 hours (or 2 days) before the person has any symptoms or tests positive. By informing your close contacts they may have been exposed, you are helping to protect everyone.

If you think your positive test result may be incorrect, contact a healthcare provider to determine whether or not additional testing is necessary.

 **Watch Video:** [How to interpret self test results \[00:01:32\]](#) 

If Your Test Result Is Negative

A negative test result means the virus that causes COVID-19 was not found in your specimen, and you may have a lower risk of transmitting the disease to others. If you took the test while you had symptoms and followed all instructions carefully, a negative result means your current illness is probably not COVID-19, though it does not rule out a COVID-19 infection.

However, it is possible for a test to give a negative result in some people who have COVID-19. This is called a false negative. You could also test negative if the specimen was collected too early in your infection. In this case, you could test positive later during your illness.

Serial Testing (Repeat Testing)

Some self-tests are designed to be used in a series. Serial self-testing is when a person tests themselves multiple times for COVID-19, or on a routine basis, such as every few days. By testing more frequently, you might detect the virus that causes COVID-19 more quickly and could reduce the spread of infection. Some self-tests include instructions for performing serial testing, including the number of days between tests, and may include more than one test in the package.

If your self-test is negative, you should follow the manufacturer's instructions for use for serial testing, if applicable, that are included in the test box, or you can find the instructions for your test ([molecular](#)  or [antigen](#) ) on the FDA website. They may recommend you test again within 2 or 3 days. Contact a healthcare provider if you have any questions about your test results or serial testing. You may also use the [COVID-19 Viral Testing Tool](#) to help you determine the next steps after testing. If you have [COVID-19 symptoms](#), especially if you have been exposed to someone who has COVID-19, you should [quarantine according to CDC recommendations](#).

If you think your negative test result may be incorrect, contact a healthcare provider to determine whether or not additional testing is necessary.

If Your Result Shows Invalid or Error

Sometimes invalid results or an error can show on the test device. Invalid results or an error can occur for many reasons. Your specimen may not have been collected correctly, or the test may have malfunctioned.

Invalid test results are rare but can occur. If the self-test shows an invalid result or a test error, the test did not work properly. If this happens, refer to the instructions for use in the package insert and contact the manufacturer for assistance.

How to Collect an Anterior Nasal Swab Specimen for COVID-19 Testing



Use **only** an authorized specimen collection kit given to you by your healthcare provider or personnel at the testing center, or purchased over the counter in a pharmacy or other store. Follow the instructions included with the specimen collection kit which may be used on-site or at home. Use **only** materials provided in the kit to collect and store or mail the specimen, unless the kit says to do otherwise. These instructions can also be used with self-testing kits if the kit specifies anterior nasal swab collection.

Set-up

1. Disinfect the surface where you will open the collection kit. Remove and lay out contents of kit. Read instructions before starting specimen collection.



2. Wash hands with soap and water. If soap and water are not available, use hand sanitizer.



Specimen collection

3. Remove the swab from the package. Do not touch the soft end with your hands or anything else.



4. Insert the entire soft end of the swab into your nostril **no more than 3/4 of an inch** (1.5 cm) into your nose.



5. Slowly rotate the swab, gently pressing against the inside of your nostril at least 4 times for a total of 15 seconds. Get as much nasal discharge as possible on the soft end of the swab.



6. Gently remove the swab.



7. Using the same swab, repeat steps 4–6 in your other nostril with the same end of the swab.

[cdc.gov/coronavirus](https://www.cdc.gov/coronavirus)

CS-311760-C 04/13/2021

How to Collect An Anterior Nasal Swab Specimen for COVID-19 Testing

English [372 KB, 2 pages]

Español [395 KB, 2 pages]

How to Collect a Nasal Mid-Turbinate Specimen for COVID-19 Testing



Use **only** an authorized specimen collection kit. Get a kit from your healthcare provider or a testing center. Kits are available at a pharmacy or other retail outlet. Follow the instructions included with the specimen collection kit. Use **only** materials provided in the kit to collect and store or mail the specimen unless the kit says to do otherwise. Use these instructions with kits that specify a nasal mid-turbinate swab collection.

Setup

1. Disinfect the surface where you will open the collection kit. Remove and lay out contents of kit. Read instructions before starting specimen collection.



2. Wash hands with soap and water. If soap and water are not available, use hand sanitizer.



Specimen Collection

3. Remove the swab from the package. Do not touch the soft end with your hands or anything else.



4. Insert the entire soft end of the swab straight back into your nostril **less than one inch** (about 2cm) or until you feel resistance.



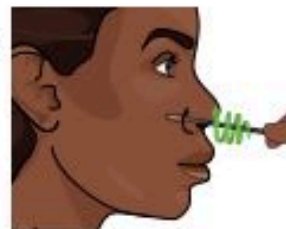
5. Slowly rotate the swab, gently rubbing it along the insides of your nasal passage several times.



6. Gently remove the swab.



7. Using the same swab, repeat steps 4–6 in your other nostril with the same end of the swab.



[cdc.gov/coronavirus](https://www.cdc.gov/coronavirus)

How to Collect a Nasal Mid-Turbinate Swab Specimen for COVID-19 Testing

English [4 MB, 2 pages]

Español [4 MB, 2 pages]

USING YOUR SELF-TEST | COVID-19 |

1

Follow instructions very carefully.



2

Use the QR code below for more information about self-testing and how-to videos.



3

If your results are positive

Isolate yourself from others. As much as possible, stay in a specific room and away from other people and pets in your home.

Tell your close contacts that they may have been exposed to COVID-19.



4



Call your health care provider with any questions, including if you have worsening COVID-19 symptoms.



www.cdc.gov/covidtesting

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 [PDF - 295 KB, 1 page]

Using Your Self-Test

English  [295 KB, 1 page]

USING YOUR SELF-TEST | COVID-19 |

1 Follow instructions very carefully.



Use the QR code below for more information about self-testing and how-to videos.

2



3

If your results are positive
Isolate yourself from others. As much as possible, stay in a specific room and away from other people and pets in your home.

Tell your close contacts that they may have been exposed to COVID-19.



4

Call your health care provider with any questions, including if you have worsening COVID-19 symptoms. You can contact
[insert name of provider⁺]
at the number below

[Insert phone number⁺]



www.cdc.gov/covidtesting

COVID-19 101/2/3

[PDF - 272 KB, 1 Page]

Using Your Self-Test - customizable

English [272 KB, 1 page]

Chinese [338 KB, 1 page]

French [319 KB, 1 page]

Previous Updates

Updates from Previous Content



As of November 4, 2021

- Self-testing videos are now available to assist with how to use and interpret self-tests.
- Removed recommendation to report test results to healthcare providers or health departments.
- Added guidance about seeking assistance from a healthcare provider if the test result seems incorrect.

As of October 21, 2021

- Based on evolving evidence, CDC recommends fully vaccinated people get tested 5-7 days after close contact with a person with suspected or confirmed COVID-19.

As of August 2, 2021

- Added a recommendation for fully vaccinated people who have a known exposure to someone with suspected or confirmed COVID-19 to be tested 3–5 days after exposure, and to wear a mask in public indoor settings for 14 days or until they receive a negative test result.
- Added language about quarantining for fully vaccinated and unvaccinated people.

As of July 21, 2021

- Added language about people with symptoms of COVID-19 being tested regardless of vaccination status.
- Expanded language on what to do if your self-test result is negative.
- Included a link to the new COVID-19 Viral Testing Tool.

Last Updated Dec. 29, 2021

In Vitro Diagnostics EUAs - Antigen Diagnostic Tests for SARS-CoV-2

In Vitro Diagnostic EUAs: Overview and Templates

- [Templates and Other Information \(/web/20220124170354/https://www.fda.gov/medical-devices/coronavirus-disease-2019-covid-19-emergency-use-authorizations-medical-devices/in-vitro-diagnostics-euas\)](https://web/20220124170354/https://www.fda.gov/medical-devices/coronavirus-disease-2019-covid-19-emergency-use-authorizations-medical-devices/in-vitro-diagnostics-euas)
- [Molecular Diagnostic Tests for SARS-CoV-2 \(/web/20220124170354/https://www.fda.gov/medical-devices/coronavirus-disease-2019-covid-19-emergency-use-authorizations-medical-devices/in-vitro-diagnostics-euas-molecular-diagnostic-tests-sars-cov-2\)](https://web/20220124170354/https://www.fda.gov/medical-devices/coronavirus-disease-2019-covid-19-emergency-use-authorizations-medical-devices/in-vitro-diagnostics-euas-molecular-diagnostic-tests-sars-cov-2)
- [Serology and Other Adaptive Immune Response Tests for SARS-CoV-2 \(/web/20220124170354/https://www.fda.gov/medical-devices/coronavirus-disease-2019-covid-19-emergency-use-authorizations-medical-devices/in-vitro-diagnostics-euas-serology-and-other-adaptive-immune-response-tests-sars-cov-2\)](https://web/20220124170354/https://www.fda.gov/medical-devices/coronavirus-disease-2019-covid-19-emergency-use-authorizations-medical-devices/in-vitro-diagnostics-euas-serology-and-other-adaptive-immune-response-tests-sars-cov-2)
- [IVDs for Management of COVID-19 Patients \(/web/20220124170354/https://www.fda.gov/medical-devices/coronavirus-disease-2019-covid-19-emergency-use-authorizations-medical-devices/in-vitro-diagnostics-euas-ivds-management-covid-19-patients\)](https://web/20220124170354/https://www.fda.gov/medical-devices/coronavirus-disease-2019-covid-19-emergency-use-authorizations-medical-devices/in-vitro-diagnostics-euas-ivds-management-covid-19-patients)

On This Page:

- [Individual EUAs for Antigen Diagnostic Tests for SARS-CoV-2](#)
- [Revision Concerning Viral Mutations](#)

Individual EUAs for Antigen Diagnostic Tests for SARS-CoV-2

This table includes information about authorized SARS-CoV-2 antigen diagnostic tests that have been authorized individually. These EUAs have been issued for each individual test with certain conditions of authorization required of the manufacturer and authorized laboratories. Test attributes are listed in the "Attributes" column. For example, tests authorized for the screening of asymptomatic individuals without known exposure are listed with "screening" in the attribute column; pooling, multi-analyte, saliva, home collection, and home testing are similarly listed. Tests available without a prescription include the attribute "DTC" (for direct-to-consumer home collection tests) or "OTC" (for over-the-counter at-home tests).

Tests with "single target" in the attribute column are:

- designed to detect only one antigen target, currently either a section of the spike protein or a section of the nucleocapsid protein;
- more susceptible to changes in performance due to viral mutations, meaning they are more likely to fail to detect new variants.

Tests with "multiple targets" in the attribute column are:

- designed to detect more than one section of the proteins that make up SARS-CoV-2;
- more likely to continue to perform as labeled as new variants emerge.

To see additional authorization documents, such as letters granting EUA amendments or revisions, and a list of other brand names authorized under a specific EUA, select the plus (+) button beside the "Date EUA Issued or Last Updated" for each EUA.

Authorized EUA devices in the table below for SARS-CoV-2 antigen diagnostic tests are assigned the QKP product code. Antigen multi-analyte respiratory panel tests authorized in the table below are assigned the QMN product code.

Search:

Show entries

Date EUA Issued or Last Updated	Entity	Diagnostic (Most Recent Letter of Authorization) and Date EUA Originally Issued	Attributes
+ 01/22/2022	Access Bio, Inc.	CareStart COVID-19 Antigen Home Test (/web/20220124170354/https://www.fda.gov/media/151245/download) 08/02/2021	Lateral Flow, Visual Read, Over the Counter (OTC) Home Testing, Serial Screening, Single Target

Date EUA Issued or Last Updated	Entity	Diagnostic (Most Recent Letter of Authorization) and Date EUA Originally Issued	Attributes
+ 01/19/2022	Maxim Biomedical, Inc.	<u>MaximBio ClearDetect COVID-19 Antigen Home Test (/web/20220124170354/https://www.fda.gov/media/155632/download)</u> 01/19/2022	Lateral Flow, Visual Read, Over the Counter (OTC) Home Testing, Serial Screening, Single Target
+ 01/14/2022	PHASE Scientific International, Ltd.	<u>INDICAID COVID-19 Rapid Antigen Test (/web/20220124170354/https://www.fda.gov/media/151212/download)</u> 07/28/2021	Lateral Flow, Visual Read, Serial Screening, Single Target
+ 01/14/2022	iHealth Labs, Inc.	<u>iHealth COVID-19 Antigen Rapid Test Pro (/web/20220124170354/https://www.fda.gov/media/155487/download)</u> 01/14/2022	Lateral Flow, Visual Read, Serial Screening, Single Target
+ 01/07/2022	Abbott Diagnostics Scarborough, Inc.	<u>BinaxNOW COVID-19 Ag Card (/web/20220124170354/https://www.fda.gov/media/141567/download)</u> 08/26/2020	Lateral Flow, Visual Read, Single Target
+ 01/07/2022	Abbott Diagnostics Scarborough, Inc.	<u>BinaxNOW COVID-19 Ag Card Home Test (/web/20220124170354/https://www.fda.gov/media/144576/download)</u> 12/16/2020	Lateral Flow, Visual Read, Prescription Home Testing, Telehealth Proctor Supervised , Single Target
+ 01/07/2022	Abbott Diagnostics Scarborough, Inc.	<u>BinaxNOW COVID-19 Antigen Self Test (/web/20220124170354/https://www.fda.gov/media/147251/download)</u> 03/31/2021	Lateral Flow, Visual Read, Over the Counter (OTC) Home Testing, Serial Screening, Single Target
+ 01/07/2022	Abbott Diagnostics Scarborough, Inc.	<u>BinaxNOW COVID-19 Ag Card 2 Home Test (/web/20220124170354/https://www.fda.gov/media/147256/download)</u> 03/31/2021	Lateral Flow, Visual Read, Over the Counter (OTC) Home Testing, Telehealth Proctor Supervised, Serial Screening, Single Target
+ 01/07/2022	Abbott Diagnostics Scarborough, Inc.	<u>BinaxNOW COVID-19 Ag 2 Card (/web/20220124170354/https://www.fda.gov/media/147261/download)</u> 03/31/2021	Lateral Flow, Visual Read, Non-prescription Testing, Serial Screening, Single Target
+ 01/05/2022	SD Biosensor, Inc.	<u>COVID-19 At-Home Test (/web/20220124170354/https://www.fda.gov/media/155123/download)</u> 12/24/2021	Lateral Flow, Visual Read, Over the Counter (OTC) Home Testing, Serial Screening, Single Target
+ 12/29/2021	Siemens Healthineers	<u>CLINITEST Rapid COVID-19 Antigen Self-Test (/web/20220124170354/https://www.fda.gov/media/155173/download)</u> 12/29/2021	Lateral Flow, Visual Read, Over the Counter (OTC) Home Testing, Serial Screening, Single Target

Date EUA Issued or Last Updated	Entity	Diagnostic (Most Recent Letter of Authorization) and Date EUA Originally Issued	Attributes
+ 12/22/2021	iHealth Labs, (/web/20220124170354/https://www.fda.gov/media/153925/download)Inc.	iHealth COVID-19 Antigen Rapid Test (/web/20220124170354/https://www.fda.gov/media/153925/download) 11/05/2021	Lateral Flow, Visual Read, Over the Counter (OTC) Home Testing, Serial Screening, Single Target
+ 12/21/2021	Quanterix Corporation	Simoa SARS-CoV-2 N Protein Antigen Test (/web/20220124170354/https://www.fda.gov/media/144925/download) 01/05/2021	Paramagnetic Microbead-based Immunoassay, Serial Screening, Saliva, Single Target
+ 12/17/2021	Salofa Oy	Sienna-Clarity COVID-19 Antigen Rapid Test Cassette (/web/20220124170354/https://www.fda.gov/media/149056/download) 05/20/2021	Lateral Flow, Visual Read, Single Target
+ 12/10/2021	Becton, Dickinson and Company (BD)	BD Veritor System for Rapid Detection of SARS-CoV-2 (/web/20220124170354/https://www.fda.gov/media/139752/download) 07/02/2020	Chromatographic Digital Immunoassay, Instrument Read, Serial Screening, Single Target
+ 12/06/2021	Nano-Ditech Corp.	Nano-Check COVID-19 Antigen Test (/web/20220124170354/https://www.fda.gov/media/154715/download) 12/06/2021	Lateral Flow, Visual Read, Serial Screening, Single Target
+ 12/02/2021	Access Bio, Inc.	CareStart COVID-19 Antigen test (/web/20220124170354/https://www.fda.gov/media/142916/download) 10/08/2020	Lateral Flow, Visual Read, Serial Screening, Single Target
+ 11/23/2021	Becton, Dickinson and Company (BD)	BD Veritor At-Home COVID-19 Test (/web/20220124170354/https://www.fda.gov/media/151771/download) 8/24/2021	Lateral Flow, Digital Read, Over the Counter (OTC) Home Testing, Serial Screening, Single Target
+ 11/22/2021	InBios International Inc.	SCoV-2 Ag Detect Rapid Self-Test (/web/20220124170354/https://www.fda.gov/media/154338/download) 11/22/2021	Lateral Flow, Visual Read, Over the Counter (OTC) Home Testing, Serial Screening, Single Target
+ 11/17/2021	GenBody Inc.	GenBody COVID-19 Ag (/web/20220124170354/https://www.fda.gov/media/150785/download) 07/13/2021	Lateral Flow, Visual Read, Serial Screening, Single Target
+ 11/16/2021	Ortho Clinical Diagnostics, Inc.	VITROS Immunodiagnostic Products SARS-CoV-2 Antigen Reagent Pack (/web/20220124170354/https://www.fda.gov/media/145069/download) 01/11/2021	Chemiluminescence Immunoassay, Instrument Read, Single Target

Date EUA Issued or Last Updated	Entity	Diagnostic (Most Recent Letter of Authorization) and Date EUA Originally Issued	Attributes
+ 11/09/2021	Quidel Corporation	QuickVue SARS Antigen Test (/web/20220124170354/https://www.fda.gov/media/144665/download) 12/18/2020	Lateral Flow, Visual Read, Serial Screening, Single Target
+ 11/01/2021	OraSure Technologies, Inc.	InteliSwab COVID-19 Rapid Test (/web/20220124170354/https://www.fda.gov/media/149908/download) 06/04/2021	Lateral Flow, Visual Read, Over the Counter (OTC) Home Testing, Serial Screening, Single Target
+ 10/29/2021	LumiraDx UK Ltd.	LumiraDx SARS-CoV-2 Ag Test (/web/20220124170354/https://www.fda.gov/media/141301/download) 08/18/2020	Microfluidic Immunofluorescence Assay, Instrument Read, Screening, Single Target
+ 10/27/2021	Princeton BioMeditech Corp.	Status COVID-19/Flu A&B (/web/20220124170354/https://www.fda.gov/media/145694/download) 02/04/2021	Lateral Flow, Visual Read, Multi-analyte, Single Target

Showing 1 to 25 of 45 entries

¹ Authorized settings include the following:

- H - Laboratories certified under the Clinical Laboratory Improvement Amendments of 1988 (CLIA), 42 U.S.C. §263a, that meet requirements to perform high complexity tests.
- M - Laboratories certified under the Clinical Laboratory Improvement Amendments of 1988 (CLIA), 42 U.S.C. §263a, that meet requirements to perform moderate complexity tests.
- W - Patient care settings operating under a CLIA Certificate of Waiver.

² Authorization Documents include the Healthcare Provider (HCP) and Patient Fact Sheets and either the Manufacture Instructions/Package Insert (abbreviated to IFU) or the EUA Summary.

Revision Concerning Viral Mutations

On September 23, 2021, the FDA revised the EUAs of certain molecular, antigen, and serology tests to establish additional Conditions of Authorization in response to the continued emergence of new variants of SARS-CoV-2. The revision requires test developers to update their authorized labeling and evaluate the impact of SARS-CoV-2 viral mutations on their test's performance as outlined in the letter. This revision is effective as of September 23, 2021 for all EUAs that are within the scope of the revision. This revision does not apply to EUAs for authorized IL-6 assays or standalone specimen collection devices and does not apply to EUAs that include substantially equivalent viral mutation conditions of authorization.

The FDA has determined that establishing additional conditions is necessary to mitigate the potential risk of false negative results due to either decreased sensitivity or non-reactivity associated with SARS-CoV-2 viral mutations. As set forth in the September 23, 2021 letter, developers of authorized tests that are within the scope of the revision are now required to routinely monitor emerging viral mutations and their potential impact on the performance of the authorized SARS-CoV-2 test(s). If potential impacts are identified, the EUA holder must communicate with the FDA and end users about the potential risk that presence of the mutations may have on test performance. The EUA holder must also update their authorized labeling consistent with the revision letter and submit the labeling to the FDA within 3 months of September 23, 2021. By taking these steps, the FDA and the test developer can quickly act in response to the potential risks identified and, when applicable, share the findings on [SARS-CoV-2 Viral Mutations: Impact on COVID-19 Tests](#). (https://web.archive.org/web/20220124170354/https://www.fda.gov/medical-devices/coronavirus-covid-19-and-medical-devices/sars-cov-2-viral-mutations-impact-covid-19-tests?utm_medium=email&utm_source=govdelivery)

[Viral Mutation Revision Letter – September 23, 2021](#) ([/web/20220124170354/https://www.fda.gov/media/152406/download](#))



COVID-19

We have the tools to
Fight Omicron



Vaccines & Booster



Masks



Testing

How to Protect Yourself & Others

Updated Jan. 20, 2022

Get Vaccinated and stay up to date on your COVID-19 vaccines

- [COVID-19 vaccines](#) are effective at preventing you from getting sick. [COVID-19 vaccines](#) are highly effective at preventing severe illness, hospitalizations, and death.
- Getting vaccinated is the best way to slow the spread of SARS-CoV-2, the virus that causes COVID-19
- CDC recommends that everyone who is eligible [stay up to date on their COVID-19 vaccines](#), including [people with weakened immune systems](#).

Wear a mask

- Everyone ages 2 years and older should properly wear a [well-fitting mask](#) indoors in public in areas of [substantial or high community transmission](#), regardless of vaccination status.
 - You might choose to wear a mask regardless of the level of community transmission, if you or someone in your household is [at increased risk for severe disease](#) or has a [weakened immune system](#), or if someone in your household is not [up to date](#) on their COVID-19 vaccines or not eligible to receive COVID-19 vaccines.
- Improve how well your mask [protects](#) you, and learn about how to [choose a mask](#) to protect yourself and others.
- In general, people do not need to wear masks when outdoors. In areas of [substantial or high transmission](#), people might choose to wear a mask outdoors when in sustained [close contact](#) with other people, particularly if
 - They or someone they live with has a [weakened immune system](#) or is at [increased risk for severe disease](#).
 - They are not [up to date](#) on COVID-19 vaccines or live with someone who is not up to date on COVID-19 vaccines.
- People who have a condition or are taking medications that [weaken their immune system](#) may not be fully protected even if they are up to date on their COVID-19 vaccines. They should continue to take all [precautions recommended for unvaccinated people, including properly wearing a well-fitted mask](#), until advised otherwise by their healthcare provider.
 - For more information, see [COVID-19 Vaccines for Moderately or Severely Immunocompromised People](#)

[Wearing a mask over your nose and mouth is required](#) on planes, buses, trains, and other forms of public transportation traveling into, within, or out of the United States and while indoors at U.S. transportation hubs such as airports and stations. Travelers are not required to wear a mask in outdoor areas of a conveyance (like on open deck areas of a ferry or the uncovered top deck of a bus).

Stay 6 feet away from others

- Inside your home: Avoid close contact with people who are sick, if possible. If possible, maintain 6 feet between the person who is sick and other household members. If you are taking care of someone who is sick, make sure you properly wear a [well-fitting mask](#) and follow other steps to protect yourself.
- Indoors in public: If you are not [up to date on COVID-19 vaccines](#), stay at least 6 feet away from other people, especially if you are at [higher risk of getting very sick](#) with COVID-19.

Avoid poorly ventilated spaces and crowds

- If indoors, [bring in fresh air](#) by opening windows and doors, if possible.
- If you are at [increased risk of getting very sick](#) from COVID-19, avoid crowded places and indoor spaces that do not have fresh air from the outdoors.

Test to prevent spread to others

- You can choose from many different [types of tests](#).
- Tests for [SARS-CoV-2](#)(the virus that causes COVID-19) tell you [if you have an infection](#) at the time of the test. This type of test is called a **viral test** because it looks for viral infection.
- Regardless of the test type you select, a [positive test result](#) means that you have an infection and should [isolate](#) and inform your [close contacts](#) to avoid spreading disease to others.
- Over-the-counter [self-tests](#) are viral tests that can be used at home or anywhere, are easy to use, and produce rapid results. Anyone can use self-tests, regardless of their vaccination status or whether they have symptoms.
- COVID-19 [self-tests](#) are one of many risk-reduction measures, along with vaccination, masking, and physical distancing, that protect you and others by reducing the chances of spreading COVID-19.

Wash your hands often

- [Wash your hands](#) often with soap and water for at least 20 seconds especially after you have been in a public place, or after blowing your nose, coughing, or sneezing.
- It's especially important to wash your hands:
 - Before eating or preparing food
 - Before touching your face
 - After using the restroom
 - After leaving a public place
 - After blowing your nose, coughing, or sneezing
 - After handling your mask
 - After changing a diaper
 - After caring for someone sick
 - After touching animals or pets
- If soap and water are not readily available, **use a hand sanitizer that contains at least 60% alcohol**. Cover all surfaces of your hands and rub them together until they feel dry.
- **Avoid touching your eyes, nose, and mouth** with unwashed hands.

Cover coughs and sneezes

- **If you are wearing a mask:** You can cough or sneeze into your mask. Put on a new, clean mask as soon as possible and wash your hands.
- **If you are not wearing a mask:**

- Always cover your mouth and nose with a tissue when you cough or sneeze, or use the inside of your elbow and do not spit.
- Throw used tissues in the trash.
- Immediately **wash your hands** with soap and water for at least 20 seconds. If soap and water are not readily available, clean your hands with a hand sanitizer that contains at least 60% alcohol.

Clean and disinfect

- Clean high touch surfaces regularly or as needed and after you have visitors in your home. This includes tables, doorknobs, light switches, countertops, handles, desks, phones, keyboards, toilets, faucets, and sinks.
- If someone is sick or has tested positive for COVID-19, disinfect [frequently touched surfaces](#).
 - Use a household disinfectant product from [EPA's List N: Disinfectants for Coronavirus \(COVID-19\)](#) [↗](#) according to manufacturer's labeled directions.
 - If surfaces are dirty, clean them using detergent or soap and water prior to disinfection.

Monitor your health daily

- Be alert for symptoms:
 - Watch for fever, cough, shortness of breath, or [other symptoms of COVID-19](#).
 - Take your temperature if symptoms develop.
 - Don't take your temperature within 30 minutes of exercising or after taking medications that could lower your temperature, like acetaminophen.
 - Follow [CDC guidance](#) if symptoms develop.
- Monitoring symptoms is especially important if you are running errands, going into the office or workplace, and in settings where it may be difficult to keep a physical distance of 6 feet.

Follow recommendations for quarantine

- If you come into [close contact](#) with someone with COVID-19: follow CDC's [recommendations for quarantine](#).

Follow recommendations for isolation

- If you [test positive](#) for COVID-19 or have [symptoms](#): follow CDC's [recommendations for isolation](#).

Take precautions when you travel

- Follow CDC's [recommendations for domestic and international travel](#).

Additional Resources

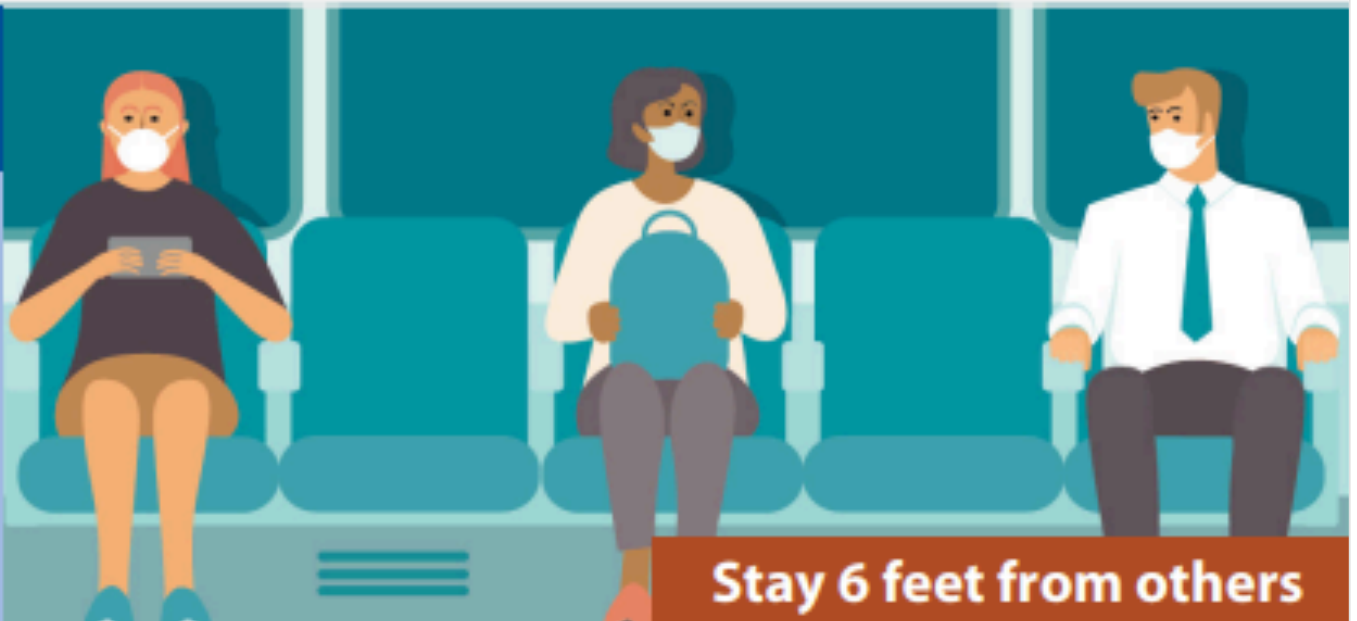
For more information, see:

- [Families with vaccinated and unvaccinated members](#)
- [Improve How Your Mask Protects You](#)
- Information for specific groups of people (link: [COVID-19 Information for Specific Groups of People | CDC](#))

Help protect yourself and others

Help Protect Yourself and Others | COVID-19

Get a COVID-19 Vaccine



Stay 6 feet from others

Avoid crowds and poorly ventilated spaces



Test to prevent spread to others



Wash your hands often



Wear a mask indoors
In areas of substantial or high transmission

cdc.gov/coronavirus

CS 3271468 | 12/13/2021

[PDF - 263 KB, 1 page]

Related Pages

- › [Prevent Getting Sick](#)
- › [Symptoms](#)

- › [How COVID-19 Spreads](#)
- › [If You Are Sick or Caring for Someone](#)
- › [People at Increased Risk](#)
- › [Frequently Asked Questions](#)
- › [Hand Sanitizer Use](#)
- › [Quarantine and Isolation](#)

Last Updated Jan. 20, 2022

In Vitro Diagnostics EUAs - Molecular Diagnostic Tests for SARS-CoV-2

In Vitro Diagnostic EUAs: Overview and Templates

- [Templates and Other Information \(/web/20220125003500/https://www.fda.gov/medical-devices/coronavirus-disease-2019-covid-19-emergency-use-authorizations-medical-devices/in-vitro-diagnostics-euas\)](https://web/20220125003500/https://www.fda.gov/medical-devices/coronavirus-disease-2019-covid-19-emergency-use-authorizations-medical-devices/in-vitro-diagnostics-euas).
- [Antigen Diagnostic Tests for SARS-CoV-2 \(/web/20220125003500/https://www.fda.gov/medical-devices/coronavirus-disease-2019-covid-19-emergency-use-authorizations-medical-devices/in-vitro-diagnostics-euas-antigen-diagnostic-tests-sars-cov-2\)](https://web/20220125003500/https://www.fda.gov/medical-devices/coronavirus-disease-2019-covid-19-emergency-use-authorizations-medical-devices/in-vitro-diagnostics-euas-antigen-diagnostic-tests-sars-cov-2).
- [Serology and Other Adaptive Immune Response Tests for SARS-CoV-2 \(/web/20220125003500/https://www.fda.gov/medical-devices/coronavirus-disease-2019-covid-19-emergency-use-authorizations-medical-devices/in-vitro-diagnostics-euas-serology-and-other-adaptive-immune-response-tests-sars-cov-2\)](https://web/20220125003500/https://www.fda.gov/medical-devices/coronavirus-disease-2019-covid-19-emergency-use-authorizations-medical-devices/in-vitro-diagnostics-euas-serology-and-other-adaptive-immune-response-tests-sars-cov-2).
- [IVDs for Management of COVID-19 Patients \(/web/20220125003500/https://www.fda.gov/medical-devices/coronavirus-disease-2019-covid-19-emergency-use-authorizations-medical-devices/in-vitro-diagnostics-euas-ivds-management-covid-19-patients\)](https://web/20220125003500/https://www.fda.gov/medical-devices/coronavirus-disease-2019-covid-19-emergency-use-authorizations-medical-devices/in-vitro-diagnostics-euas-ivds-management-covid-19-patients).

On This Page:

- [Individual EUAs for Molecular Diagnostic Tests for SARS-CoV-2](#)
- [Revision Concerning Viral Mutation](#)
- [Pooling and Serial Testing Amendment for Certain Molecular Diagnostic Tests for SARS-CoV-2](#)
- [Umbrella EUA for SARS-CoV-2 Molecular Diagnostic Tests for Serial Testing](#)
- [EUA for Molecular Diagnostic Tests for SARS-CoV-2 Developed And Performed By Laboratories Certified Under CLIA To Perform High Complexity Tests](#)

Individual EUAs for Molecular Diagnostic Tests for SARS-CoV-2

This table includes information about authorized SARS-CoV-2 molecular diagnostic tests. These EUAs have been issued for each individual test with certain conditions of authorization required of the manufacturer and authorized laboratories. Test attributes are listed in the "Attributes" column. For example, tests authorized for the screening of asymptomatic individuals without known exposure are listed with "screening" in the attribute column; pooling, multi-analyte, saliva, home collection, and home testing are similarly listed. Tests available without a prescription include the attribute "DTC" (for direct-to-consumer home collection tests) or "OTC" (for over-the-counter at-home tests). To see additional authorization documents, such as letters granting EUA amendments or revisions, and a list of other brand names authorized under a specific EUA, select the plus (+) button beside the "Date EUA Issued or Last Updated" for the EUA.

For information on tests that have been revoked see [Historical Information about Device Emergency Use Authorizations: In Vitro Diagnostics \(IVD\)](https://web.archive.org/web/20220125003500/https://www.fda.gov/medical-devices/emergency-use-authorizations-medical-devices/historical-information-about-device-emergency-use-authorizations#ivd) (<https://web.archive.org/web/20220125003500/https://www.fda.gov/medical-devices/emergency-use-authorizations-medical-devices/historical-information-about-device-emergency-use-authorizations#ivd>).

Molecular SARS-CoV-2 Diagnostic Tests for COVID-19 that have been granted a De Novo, 510(k) clearance or PMA

- BioFire Respiratory Panel 2.1 (RP2.1) - On March 17, 2021, FDA granted the first marketing authorization using the De Novo review pathway for the [BioFire Respiratory Panel 2.1 \(RP2.1\)](https://web.archive.org/web/20220125003500/https://www.accessdata.fda.gov/cdrh_docs/pdf20/DEN200031.pdf) (https://web.archive.org/web/20220125003500/https://www.accessdata.fda.gov/cdrh_docs/pdf20/DEN200031.pdf) (PDF - 630 KB). The BioFire RP2.1 is for the simultaneous qualitative detection and identification of multiple respiratory viral and bacterial nucleic acids in nasopharyngeal swabs (NPS) obtained from individuals suspected of respiratory tract infections, including COVID-19. Also see the FDA news release: [FDA Permits Marketing of First SARS-CoV-2 Diagnostic Test Using Traditional Premarket Review Process](https://web/20220125003500/https://www.fda.gov/news-events/press-announcements/fda-permits-marketing-first-sars-cov-2-diagnostic-test-using-traditional-premarket-review-process) ([/web/20220125003500/https://www.fda.gov/news-events/press-announcements/fda-permits-marketing-first-sars-cov-2-diagnostic-test-using-traditional-premarket-review-process](https://web/20220125003500/https://www.fda.gov/news-events/press-announcements/fda-permits-marketing-first-sars-cov-2-diagnostic-test-using-traditional-premarket-review-process)). With granting of the De Novo for the BioFire RP2.1, [the FDA revoked the EUA for this device \(/web/20220125003500/https://www.fda.gov/medical-devices/emergency-use-authorizations-medical-devices/historical-information-about-device-emergency-use-authorizations\)](https://web/20220125003500/https://www.fda.gov/medical-devices/emergency-use-authorizations-medical-devices/historical-information-about-device-emergency-use-authorizations), which was initially authorized for emergency use in May 2020.

The BioFire Respiratory Panel 2.1 (RP2.1) was reviewed under the [De Novo premarket review pathway](https://web/20220125003500/https://www.fda.gov/medical-devices/premarket-submissions-selecting-and-preparing-correct-submission/de-novo-classification-request) ([/web/20220125003500/https://www.fda.gov/medical-devices/premarket-submissions-selecting-and-preparing-correct-submission/de-novo-classification-request](https://web/20220125003500/https://www.fda.gov/medical-devices/premarket-submissions-selecting-and-preparing-correct-submission/de-novo-classification-request)), a regulatory pathway for low-to-moderate-risk devices of a new type. Along with this De Novo authorization, the FDA is establishing criteria, called special controls, that define the requirements related to labeling and performance testing. When

met, the special controls, in combination with general controls, provide a reasonable assurance of safety and effectiveness for tests of this type. This action also creates a new regulatory classification, which means that subsequent devices of the same type with the same intended use may go through the FDA's 510(k) pathway, whereby devices can obtain clearance by demonstrating substantial equivalence to a predicate device.

Tests with "single target" in the attribute column are:

- designed to detect only one viral target on the SARS-CoV-2 genome;
- more susceptible to changes in performance due to viral mutations, meaning they are more likely to fail to detect new variants.

Tests with "multiple targets" in the attribute column are:

- designed to detect two or more viral targets, or sections of the SARS-CoV-2 genome;
- more likely to continue to perform well when new variants emerge.

EUAs for standalone specimen collection devices, including home collection kits which do not include an assay in the EUA, will not have a target designation in the attribute column.

Authorized Molecular Diagnostic Tests for SARS-CoV-2 are assigned the QJR product code.

Authorized Home Collection Kits are assigned the QLV product code. Authorized EUAs for Multi-analyte Respiratory Panel Tests are assigned the QLT product code.

Search:

Show entries

Date EUA Issued or Last Updated	Entity	Diagnostic (Most Recent Letter of Authorization) and Date EUA Original Issue	Attributes ²	Authorized Setting(s) ¹	Authorization Documents ²
+ 01/21/2022	PerkinElmer, Inc.	PKamp Respiratory SARS-CoV-2 RT-PCR Panel 1 (/web/20220125003500/https://www.fda.gov/media/152969/download) 10/06/2021	Real-time RT-PCR, Multi-analyte, Multiple Targets	H	HCP (/web/20220125003500/https://www.fda.gov/media/152969/download) (/web/20220125003500/https://www.fda.gov/media/152969/download) (/web/20220125003500/https://www.fda.gov/media/152969/download)
+ 01/21/2022	National Jewish Health	SARS-CoV-2 MassArray Test (/web/20220125003500/https://www.fda.gov/media/142545/download) 09/29/2020	RT-PCR and MALDI-TOF Mass Spec. , Multiple Targets	H	HCP (/web/20220125003500/https://www.fda.gov/media/142545/download) (/web/20220125003500/https://www.fda.gov/media/142545/download) (/web/20220125003500/https://www.fda.gov/media/142545/download)
+ 01/21/2022	Mammoth Biosciences, Inc.	DETECTR BOOST SARS-CoV-2 Reagent Kit (/web/20220125003500/https://www.fda.gov/media/155637/download) 01/21/2022	RT-LAMP, CRISPR, Single Target	H	HCP (/web/20220125003500/https://www.fda.gov/media/155637/download) (/web/20220125003500/https://www.fda.gov/media/155637/download) (/web/20220125003500/https://www.fda.gov/media/155637/download)
+ 01/20/2022	DiaCarta, Inc.	QuantiVirus SARS-CoV-2 Multiplex Test Kit (/web/20220125003500/https://www.fda.gov/media/140355/download) 07/21/2020	Real-time RT-PCR, Single Target	H	HCP (/web/20220125003500/https://www.fda.gov/media/140355/download) (/web/20220125003500/https://www.fda.gov/media/140355/download) (/web/20220125003500/https://www.fda.gov/media/140355/download)
+ 01/19/2022	Fast Track Diagnostics Luxembourg S.à.r.l. (a Siemens Healthineers Company)	FTD SARS-CoV-2 (/web/20220125003500/https://www.fda.gov/media/137687/download) 05/05/2020	Real-time RT-PCR, Multiple Targets	H	HCP (/web/20220125003500/https://www.fda.gov/media/137687/download) (/web/20220125003500/https://www.fda.gov/media/137687/download) (/web/20220125003500/https://www.fda.gov/media/137687/download)
+ 01/19/2022	Illumina, Inc.	Illumina COVIDSeq Test (/web/20220125003500/https://www.fda.gov/media/138778/download) 06/09/2020	Sequencing, Multiple Targets	H	HCP (/web/20220125003500/https://www.fda.gov/media/138778/download) (/web/20220125003500/https://www.fda.gov/media/138778/download) (/web/20220125003500/https://www.fda.gov/media/138778/download)

Date EUA Issued or Last Updated	Entity	Diagnostic (Most Recent Letter of Authorization) and Date EUA Original Issue	Attributes ³	Authorized Setting(s) ¹	Authorization Documents ²
+ 01/19/2022	Viracor Eurofins Clinical Diagnostics	Viracor SARS-CoV-2 assay DTC (/web/20220125003500/https://www.fda.gov/media/146322/download), 02/26/2021	Direct to Consumer (DTC), Real- time RT-PCR, Home Collection, Screening, Multiple Targets	H	HCP (/web/20220125003500/https://www.fda.gov/media/146322/download)
+ 01/14/2022	University of Illinois Office of the Vice President for Economic Development and Innovation	covidSHIELD (/web/20220125003500/https://www.fda.gov/media/146314/download), 02/24/2021	Real-time RT- PCR, Saliva, Serial Screening, Home Collection, Pooling, Multiple Targets	H	HCP (/web/20220125003500/https://www.fda.gov/media/146314/download)
+ 01/13/2022	Kaiser Permanente Mid-Atlantic States	KPMAS COVID-19 Test (/web/20220125003500/https://www.fda.gov/media/139066/download), 06/13/2020	Real-time RT- PCR, Home Collection, Screening, Multiple Targets	H	HCP (/web/20220125003500/https://www.fda.gov/media/139066/download)
+ 01/12/2022	Detect, Inc.	Detect Covid-19 Test (/web/20220125003500/https://www.fda.gov/media/153663/download), 10/28/2021	RT, LAMP, Lateral Flow Strips, Over the Counter (OTC) Home Testing, Serial Screening, Single Target	Home, H, M, W	HCP (/web/20220125003500/https://www.fda.gov/media/153663/download)
+ 01/12/2022	Viracor Eurofins Clinical Diagnostics	Viracor SARS-CoV-2 assay (/web/20220125003500/https://www.fda.gov/media/143070/download), 04/06/2020	Real-Time RT- PCR, Pooling, Screening, Pooled Serial Screening - Swab, Multiple Targets	H	HCP (/web/20220125003500/https://www.fda.gov/media/143070/download)
+ 01/11/2022	SEASUN BIOMATERIALS	U-TOP COVID-19 Detection Kit (/web/20220125003500/https://www.fda.gov/media/137420/download), 04/27/2020	Real-time RT- PCR, Multiple Targets	H	HCP (/web/20220125003500/https://www.fda.gov/media/137420/download)
+ 01/11/2022	DiaCarta, Inc	QuantiVirus SARS-CoV-2 Test kit (/web/20220125003500/https://www.fda.gov/media/136806/download), 04/08/2020	Real-time RT- PCR, Multiple Targets	H	HCP (/web/20220125003500/https://www.fda.gov/media/136806/download)
+ 01/11/2022	SEASUN BIOMATERIALS, Inc.	AQ-TOP COVID-19 Rapid Detection Kit PLUS (/web/20220125003500/https://www.fda.gov/media/142797/download), 10/05/2020	RT-LAMP, Multiple Targets	H	HCP (/web/20220125003500/https://www.fda.gov/media/142797/download)
+ 01/07/2022	Premier Medical Laboratory Services	PMLS SARS-CoV-2 Assay (/web/20220125003500/https://www.fda.gov/media/155317/download), 01/07/2022	Real-time RT- PCR, Home Collection, Screening, Multiple Targets	H	HCP (/web/20220125003500/https://www.fda.gov/media/155317/download)
+ 01/06/2022	PreciGenome LLC	FastPlex Triplex SARS-CoV-2 detection kit (RT-Digital PCR) (/web/20220125003500/https://www.fda.gov/media/139524/download), 06/25/2020	RT-digital droplet PCR, Multiple Targets	H	HCP (/web/20220125003500/https://www.fda.gov/media/139524/download)

Date EUA Issued or Last Updated	Entity	Diagnostic (Most Recent Letter of Authorization) and Date EUA Original Issue	Attributes ³	Authorized Setting(s) ¹	Authorization Documents ²
+ 01/06/2022	George Washington University Public Health Laboratory	GWU SARS-CoV-2 RT-PCR Test (/web/20220125003500/https://www.fda.gov/media/140977/download) 08/07/2020	Real-time RT-PCR, Multiple Targets	H	HCP (/web/20220125003500/https://www.fda.gov/media/140977/download) (/web/20220125003500/https://www.fda.gov/media/140977/download) (/web/20220125003500/https://www.fda.gov/media/140977/download) (/web/20220125003500/https://www.fda.gov/media/140977/download)
+ 01/06/2022	Roche Molecular Systems, Inc.	cobas SARS-CoV-2 & Influenza A/B Nucleic Acid Test for use on the cobas Liat System (/web/20220125003500/https://www.fda.gov/media/142190/download) 09/14/2020	Real-time RT-PCR, Multi-analyte, Multiple Targets	H, M, W	HCP (/web/20220125003500/https://www.fda.gov/media/142190/download) (/web/20220125003500/https://www.fda.gov/media/142190/download) (/web/20220125003500/https://www.fda.gov/media/142190/download) (/web/20220125003500/https://www.fda.gov/media/142190/download)
+ 01/05/2022	Seasun Biomaterials, Inc.	AQ-TOP COVID-19 Rapid Detection Kit (/web/20220125003500/https://www.fda.gov/media/138300/download) 05/21/2020	RT-LAMP, Single Target	H	HCP (/web/20220125003500/https://www.fda.gov/media/138300/download) (/web/20220125003500/https://www.fda.gov/media/138300/download) (/web/20220125003500/https://www.fda.gov/media/138300/download) (/web/20220125003500/https://www.fda.gov/media/138300/download)
+ 01/04/2022	KogeneBiotech Co., Ltd.	PowerChek 2019-nCoV Real-time PCR Kit (/web/20220125003500/https://www.fda.gov/media/140066/download) 07/13/2020	Real-time RT-PCR, Multiple Targets	H	HCP (/web/20220125003500/https://www.fda.gov/media/140066/download) (/web/20220125003500/https://www.fda.gov/media/140066/download) (/web/20220125003500/https://www.fda.gov/media/140066/download) (/web/20220125003500/https://www.fda.gov/media/140066/download)
+ 01/04/2022	Roche Molecular Systems	cobas SARS-CoV-2 Nucleic acid test for use on the cobas Liat System (cobas SARS-CoV-2) (/web/20220125003500/https://www.fda.gov/media/150274/download) 06/17/2021	Real-time RT-PCR, Screening, Multiple Targets	H, M, W	HCP (/web/20220125003500/https://www.fda.gov/media/150274/download) (/web/20220125003500/https://www.fda.gov/media/150274/download) (/web/20220125003500/https://www.fda.gov/media/150274/download) (/web/20220125003500/https://www.fda.gov/media/150274/download)
+ 12/22/2021	Tide Laboratories, LLC	DTPM COVID-19 RT-PCR Test (/web/20220125003500/https://www.fda.gov/media/138815/download) 06/10/2020	Real-time RT-PCR, Single Target	H	HCP (/web/20220125003500/https://www.fda.gov/media/138815/download) (/web/20220125003500/https://www.fda.gov/media/138815/download) (/web/20220125003500/https://www.fda.gov/media/138815/download) (/web/20220125003500/https://www.fda.gov/media/138815/download)
+ 12/17/2021	UCSD BCG EXCITE Lab	UCSD EXCITE COVID-19 Test (/web/20220125003500/https://www.fda.gov/media/155007/download)12/17/2021	Real-time RT-PCR, Home Collection, Screening, Multiple Targets	H	HCP (/web/20220125003500/https://www.fda.gov/media/155007/download) (/web/20220125003500/https://www.fda.gov/media/155007/download) (https://web.archive.org/web/20211217000000/https://www.fda.gov/media/155007/download) (https://web.archive.org/web/20211217000000/https://www.fda.gov/media/155007/download)
+ 12/17/2021	STS Lab Holdco (a subsidiary of Amazon.com Services LLC)	Amazon Multi-Target SARS-CoV-2 Real-Time RT-PCR Test (/web/20220125003500/https://www.fda.gov/media/151459/download) 08/11/2021	Real-time RT-PCR, Home Collection, Pooling, Screening, Multiple Targets	H	HCP (/web/20220125003500/https://www.fda.gov/media/151459/download) (/web/20220125003500/https://www.fda.gov/media/151459/download) (/web/20220125003500/https://www.fda.gov/media/151459/download) (/web/20220125003500/https://www.fda.gov/media/151459/download)
+ 12/17/2021	STS Lab Holdco (a subsidiary of Amazon.com Services LLC)	Amazon Multi-Target SARS-CoV-2 Real-Time RT-PCR DTC Test (/web/20220125003500/https://www.fda.gov/media/151455/download) 08/11/2021	Direct to Consumer (DTC), Real-time RT-PCR, Home Collection, Pooling, Screening, Multiple Targets	H	HCP (/web/20220125003500/https://www.fda.gov/media/151455/download) (/web/20220125003500/https://www.fda.gov/media/151455/download) (/web/20220125003500/https://www.fda.gov/media/151455/download) (/web/20220125003500/https://www.fda.gov/media/151455/download)

Showing 1 to 25 of 269 entries

¹ Authorized settings include the following:

- H - Laboratories certified under the Clinical Laboratory Improvement Amendments of 1988 (CLIA), 42 U.S.C. §263a, that meet requirements to perform high complexity tests.
- M - Laboratories certified under the Clinical Laboratory Improvement Amendments of 1988 (CLIA), 42 U.S.C. §263a, that meet requirements to perform moderate complexity tests.

- W - Patient care settings operating under a CLIA Certificate of Waiver.

² Authorization Documents include the Healthcare Provider (HCP) and Patient Fact Sheets and either the Manufacture Instructions/Package Insert (abbreviated to IFU) or the EUA Summary.

³ Abbreviations:

- RT-PCR = reverse transcriptase polymerase chain reaction;
- RT = reverse transcriptase;
- LAMP = loop-mediated isothermal amplification;
- MALDI-TOF = Matrix Assisted Laser Desorption/Ionization - Time of Flight;
- TMA = Transcription Mediated Amplification;
- qSTAR = Selective Temperature Amplification Reaction;
- CRISPR = clustered regularly interspaced short palindromic repeats;

Revision Concerning Viral Mutations

On September 23, 2021, the FDA revised the EUAs of certain molecular, antigen, and serology tests to establish additional Conditions of Authorization in response to the continued emergence of new variants of SARS-CoV-2. The revision requires test developers to update their authorized labeling and evaluate the impact of SARS-CoV-2 viral mutations on their test's performance as outlined in the letter. This revision is effective as of September 23, 2021 for all EUAs that are within the scope of the revision. This revision does not apply to EUAs for authorized IL-6 assays or standalone specimen collection devices and does not apply to EUAs that include substantially equivalent viral mutation conditions of authorization.

The FDA has determined that establishing additional conditions is necessary to mitigate the potential risk of false negative results due to either decreased sensitivity or non-reactivity associated with SARS-CoV-2 viral mutations. As set forth in the September 23, 2021 letter, developers of authorized tests that are within the scope of the revision are now required to routinely monitor emerging viral mutations and their potential impact on the performance of the authorized SARS-CoV-2 test(s). If potential impacts are identified, the EUA holder must communicate with the FDA and end users about the potential risk that presence of the mutations may have on test performance. The EUA holder must also update their authorized labeling consistent with the revision letter and submit the labeling to the FDA within 3 months of September 23, 2021. By taking these steps, the FDA and the test developer can quickly act in response to the potential risks identified and, when applicable, share the findings on [SARS-CoV-2 Viral Mutations: Impact on COVID-19 Tests](https://www.fda.gov/medical-devices/coronavirus-covid-19-and-medical-devices/sars-cov-2-viral-mutations-impact-covid-19-tests?utm_medium=email&utm_source=govdelivery). (https://web.archive.org/web/20220125003500/https://www.fda.gov/medical-devices/coronavirus-covid-19-and-medical-devices/sars-cov-2-viral-mutations-impact-covid-19-tests?utm_medium=email&utm_source=govdelivery).

- [Viral Mutation Revision Letter - September 23, 2021](https://www.fda.gov/media/152406/download) ([/web/20220125003500/https://www.fda.gov/media/152406/download](https://web/20220125003500/https://www.fda.gov/media/152406/download)).

Pooling and Serial Testing Amendment for Certain Molecular Diagnostic Tests for SARS-CoV-2

On April 20, 2021, the FDA issued an amendment allowing certain authorized molecular diagnostic SARS-CoV-2 tests to be distributed and used to pool anterior nasal respiratory specimens from asymptomatic individuals as part of a serial testing program after developers submit a complete notification, including meeting required validation data, as set forth in the letter. Use of tests for these indications is limited to laboratories certified under the Clinical Laboratory Improvement Amendments of 1988 (CLIA), 42 U.S.C. §263a, to perform high complexity tests, except that tests authorized for use in specific named or designated high complexity laboratories can only be used in such laboratories.

This means that tests with EUAs that are amended by this authorization may be used with pooled anterior nasal specimens from individuals without known or suspected COVID-19 when such individuals are tested as part of a testing program that includes testing at regular intervals, at least once per week. The indications in each appendix (A-H) differ in the number of specimens that can be pooled (up to 3, up to 5, or up to 10) and the type of pooling that can be done (media pooling or swab pooling).

Prior to a test being distributed or used for any new indication, the developer must submit a notification to the FDA with the information required by the amendment, including self-certifying that the applicable validation has been completed. Tests will be added to Exhibit 1 once FDA confirms that the required documentation has been submitted. Please note that being added to Exhibit 1 does not necessarily mean that the FDA has reviewed the underlying validation data submitted or confirmed that the test is appropriately validated.

- [Amendment Letter](https://www.fda.gov/media/147737/download) ([/web/20220125003500/https://www.fda.gov/media/147737/download](https://web/20220125003500/https://www.fda.gov/media/147737/download)).
- [Appendix J - Sample Updated Fact Sheet for Health Care Providers](https://www.fda.gov/media/147735/download) ([/web/20220125003500/https://www.fda.gov/media/147735/download](https://web/20220125003500/https://www.fda.gov/media/147735/download)).
- [Appendix K - Sample Updated Fact Sheet for Patients](https://www.fda.gov/media/147736/download) ([/web/20220125003500/https://www.fda.gov/media/147736/download](https://web/20220125003500/https://www.fda.gov/media/147736/download)).

Exhibit 1 of the Pooling and Serial Testing Amendment

The table below includes information for the authorized RT-PCR molecular diagnostic tests for SARS-CoV-2 amended by this Pooling and Serial Testing Amendment. For authorization documents for each test, see [Individual EUAs for Molecular Diagnostic Tests for SARS-CoV-2](#) above.

Search:

Show entries

Date Added to Exhibit 1	EUA Holder	Test Name	Indication(s)*
06/24/2021	Clinical Enterprise, Inc.	Clinical Enterprise SARS-CoV-2-RT-PCR Assay	Swab: Up to 10 (Appendix C)
06/23/2021	Viracor Eurofins Clinical Diagnostics	Viracor SARS-CoV-2 assay	Swab: Up to 10 (Appendix C)
06/22/2021	Biomeme, Inc.	Biomeme SARS-CoV-2 Real-Time RT-PCR Test	Swab: Up to 10 (Appendix C) Media: Up to 5 (Appendix G)
06/15/2021	Clinical Research Sequencing Platform (CRSP), LLC at the Broad Institute of MIT and Harvard	CRSP SARS-CoV-2 Real-time Reverse Transcriptase (RT)-PCR Diagnostic Assay (Version 3)	Swab: Up to 10 (Appendix C)

Showing 1 to 4 of 4 entries

Previous **1** Next

* The indications in each appendix (A-H) differ in the number of specimens that can be pooled (up to 3, up to 5, or up to 10) and the type of pooling that can be done (media or swab pooling).

Tests authorized for Swab Pooling are authorized for:

- Qualitative detection of RNA from SARS-CoV-2 in pooled samples containing up to [3, 5, or 10, depending on the applicable appendix] individual human anterior nasal swabs placed in a single vial containing transport media after being collected by a healthcare provider (HCP) or self-collected under the supervision of an HCP from any individual, including individuals without symptoms or other reasons to suspect COVID-19, when tested at least once per week as part of a serial testing program.

This indication is authorized for use in laboratories certified under CLIA to perform high complexity tests except tests authorized for use in specific named or designated laboratories can only be used in such laboratories.

Tests authorized for Media Pooling are authorized for:

- Qualitative detection of RNA from SARS-CoV-2 in pooled samples containing aliquots of transport media from up to [3, 5, or 10, depending on the applicable appendix] individual human anterior nasal swab specimens that were collected by a healthcare provider (HCP) or self-collected under the supervision of an HCP from any individual and placed in individual vials containing transport media, including individuals without symptoms or other reasons to suspect COVID-19, when tested as part of a serial testing program including testing at least once per week.

This indication is authorized for use in laboratories certified under CLIA to perform high complexity tests except tests authorized for use in specific named or designated laboratories can only be used in such laboratories.

Umbrella EUA for SARS-CoV-2 Molecular Diagnostic Tests for Serial Testing

On November 15, 2021, the FDA issued an umbrella EUA for certain RT-PCR molecular-based tests, developed by laboratories, for detection of nucleic acid from SARS-CoV-2 from anterior nasal respiratory specimens for use as part of a serial testing program, that meet certain criteria for eligibility specified in the EUA. Under this EUA, authorized tests can be used with individual or pooled anterior nasal specimens for testing individuals, including individuals without symptoms or other epidemiological reasons to suspect COVID-19, when tested at least once per week. This means that tests authorized by this EUA may be used with individual or pooled anterior nasal respiratory specimens from individuals with or without known or suspected exposure to COVID-19 when such individuals are tested at least once per week, such as testing at regular intervals as part of a testing program implemented by schools, workplaces, or community groups. The indications in each appendix (A-K) differ in the number of specimens that can be pooled (1, up to 3, up to 5, or up to 10), the type of pooling that can be done (media pooling or swab pooling), and whether the test can be used with home collected specimens.

Use of tests authorized by this EUA is limited to use in the single laboratory that developed the authorized test and that is certified under the Clinical Laboratory Improvement Amendments of 1988 (CLIA), 42 U.S.C. §263a, and meets requirements to perform high complexity tests.

Tests will be added to Exhibit 1 of this EUA following submission of a complete notification to the FDA with the information required by the EUA, including self-certifying that the applicable validation has been completed, and confirmation by the FDA that the required documentation has been submitted. Being added to Exhibit 1 does not necessarily mean that the FDA has reviewed the underlying validation data submitted or confirmed that the test is appropriately validated.

- [EUA Letter of Authorization - Umbrella EUA for SARS-CoV-2 Molecular Diagnostic Tests for Serial Testing \(/web/20220125003500/https://www.fda.gov/media/154111/download\)](/web/20220125003500/https://www.fda.gov/media/154111/download)
- [Appendix L - Fact Sheet for Health Care Providers \(Template\) \(/web/20220125003500/https://www.fda.gov/media/154112/download\)](/web/20220125003500/https://www.fda.gov/media/154112/download)
- [Appendix M - Fact Sheet for Patients \(Template\) \(/web/20220125003500/https://www.fda.gov/media/154114/download\)](/web/20220125003500/https://www.fda.gov/media/154114/download)
- [Appendix N - Test Summary \(Template\) \(/web/20220125003500/https://www.fda.gov/media/154113/download\)](/web/20220125003500/https://www.fda.gov/media/154113/download)

Exhibit 1 of the Umbrella EUA for SARS-CoV-2 Molecular Diagnostic Tests for Serial Testing

This exhibit will include information for the RT-PCR molecular diagnostic tests for SARS-CoV-2 authorized by this umbrella EUA for SARS-CoV-2 Molecular Diagnostic Tests for Serial Testing. All tests authorized by this umbrella EUA are required to have at least two viral targets on the SARS-CoV-2 genome. Tests with multiple targets are more likely to continue to perform well when new variants emerge.

EUA for Molecular Diagnostic Tests for SARS-CoV-2 Developed And Performed By Laboratories Certified Under CLIA To Perform High Complexity Tests

On March 31, 2020, the FDA issued an umbrella EUA for molecular laboratory developed tests (LDTs) for detection of SARS-CoV-2 that meet certain criteria for eligibility described in the EUA. Under this EUA, authorized tests are authorized for use in the single laboratory that developed the authorized test and that is certified under the Clinical Laboratory Improvement Amendments of 1988 (CLIA), 42 U.S.C. §263a, that meet requirements to perform high complexity tests. Tests authorized under this umbrella EUA were added to Appendix A in the table below.

On November 15, 2021, the FDA reissued this EUA such that only tests that are listed in Appendix A are authorized for use as described in the EUA, and no additional tests will be authorized by this EUA. The reissued EUA also includes updated Conditions of Authorization and Fact Sheets to reflect the most up-to-date information.

- [EUA Letter of Authorization - Laboratories Who Have Developed a Molecular-Based Test \(LDTs\) for Coronavirus Disease 2019 \(COVID-19\) \(/web/20220125003500/https://www.fda.gov/media/136598/download\)](/web/20220125003500/https://www.fda.gov/media/136598/download)
- [Fact Sheet for Healthcare Providers \(/web/20220125003500/https://www.fda.gov/media/136599/download\)](/web/20220125003500/https://www.fda.gov/media/136599/download)
- [Fact Sheet for Patients \(/web/20220125003500/https://www.fda.gov/media/136600/download\)](/web/20220125003500/https://www.fda.gov/media/136600/download)
- See Appendix A table below for a list of tests authorized under this EUA

Appendix A: Authorized Molecular-Based Laboratory Developed Tests for Detection of Nucleic Acid from SARS-CoV-2

The table below includes information for the high complexity molecular-based laboratory developed SARS-CoV-2 assays authorized by [this EUA \(/web/20220125003500/https://www.fda.gov/media/136598/download\)](/web/20220125003500/https://www.fda.gov/media/136598/download) during the COVID-19 public health emergency.

To see additional authorization documents, such as letters granting EUA amendments or revisions, for a specific test, select the plus (+) button beside the "Date Last Updated" for each test.

All tests in this table are multiple target tests unless noted with an asterisk (*). Multiple target tests are designed to detect two or more viral targets, or sections of the SARS-CoV-2 genome. Tests with an asterisk are single target tests. Single target tests are designed to detect only one viral target on the SARS-CoV-2 genome. Tests with multiple targets are more likely to continue to perform well when new variants emerge. Tests with single targets are more susceptible to changes in performance due to viral mutations, meaning they are more likely to fail to detect new variants.

All authorized devices in the table below are assigned the QJR product code.

Search:

Show entries

Date Last Updated	Laboratory	Letter Granting Inclusion under EUA and Original Issue Date	EUA Summary
+ 04/06/2021	Biocerna	SARS-CoV-2 Test (/web/20220125003500/https://www.fda.gov/media/137451/download) 04/28/2020	EUA Summary (/web/20220125003500/https://www.fda.gov/media/137450/download)

Date Last Updated	Laboratory	Letter Granting Inclusion under EUA and Original Issue Date	EUA Summary
+ 02/03/2021	Corneum Laboratory Services	Corneum SARS-CoV-2 Assay (/web/20220125003500/https://www.fda.gov/media/138933/download) 06/12/2020	EUA Summary (/web/20220125003500/https://www.fda.gov/media/138934/download)
+ 01/29/2021	CSI Laboratories	CSI SARS-CoV-2 RT PCR Test (/web/20220125003500/https://www.fda.gov/media/138529/download) 06/01/2020	EUA Summary (/web/20220125003500/https://www.fda.gov/media/138528/download)
+ 01/28/2021	AIT Laboratories	SARS-CoV-2 Assay (/web/20220125003500/https://www.fda.gov/media/137373/download) 04/24/2020	EUA Summary (/web/20220125003500/https://www.fda.gov/media/137374/download)
+ 12/28/2020	Nationwide Children's Hospital	SARS-CoV-2 Assay (/web/20220125003500/https://www.fda.gov/media/137424/download) 04/27/2020	EUA Summary (/web/20220125003500/https://www.fda.gov/media/137423/download)
+ 12/28/2020	UTMG Pathology Laboratory	UTHSC/UCH SARS-CoV-2-RT-PCR Assay (/web/20220125003500/https://www.fda.gov/media/137654/download) 05/03/2020	EUA Summary (/web/20220125003500/https://www.fda.gov/media/137656/download)
+ 12/28/2020	Biocollections Worldwide, Inc.	Biocollections Worldwide SARS-CoV-2 Assay (/web/20220125003500/https://www.fda.gov/media/137896/download) 05/07/2020	EUA Summary (/web/20220125003500/https://www.fda.gov/media/137897/download)
+ 09/22/2020	Diatherix Eurofins Laboratory	SARS-CoV-2 PCR Test (/web/20220125003500/https://www.fda.gov/media/137256/download) 04/22/2020	EUA Summary (/web/20220125003500/https://www.fda.gov/media/137255/download)
+ 09/21/2020	Infectious Diseases Diagnostics Laboratory (IDDL), Boston Children's Hospital	Childrens-Altona-SARS-CoV-2 Assay (/web/20220125003500/https://www.fda.gov/media/136972/download) 04/14/2020	EUA Summary (/web/20220125003500/https://www.fda.gov/media/136971/download)
+ 09/21/2020	Hackensack University Medical Center (HUMC) Molecular Pathology Laboratory	CDI Enhanced COVID-19 Test (/web/20220125003500/https://www.fda.gov/media/137037/download) 04/15/2020	EUA Summary (/web/20220125003500/https://www.fda.gov/media/137036/download)
+ 09/21/2020	CirrusDx Laboratories	CirrusDx SARS-CoV-2 Assay (/web/20220125003500/https://www.fda.gov/media/137035/download) 04/15/2020	EUA Summary (/web/20220125003500/https://www.fda.gov/media/137034/download)
+ 09/14/2020	Columbia University Laboratory of Personalized Genomic Medicine	TRIPLEX CII-SARS-CoV-2 rRT-PCR TEST (/web/20220125003500/https://www.fda.gov/media/137982/download) 05/12/2020	EUA Summary (/web/20220125003500/https://www.fda.gov/media/137983/download)
+ 06/10/2020	Warrior Diagnostics, Inc.	Warrior Diagnostics SARS-CoV-2 Assay (/web/20220125003500/https://www.fda.gov/media/138791/download) 06/10/2020	EUA Summary (/web/20220125003500/https://www.fda.gov/media/138790/download)
+ 06/04/2020	Altru Diagnostics, Inc.	Altru Dx SARS-CoV-2 RT-PCR assay (/web/20220125003500/https://www.fda.gov/media/137545/download) 04/30/2020	EUA Summary (/web/20220125003500/https://www.fda.gov/media/137546/download)
+ 06/04/2020	Yale New Haven Hospital, Clinical Virology Laboratory	SARS-CoV-2 PCR test (/web/20220125003500/https://www.fda.gov/media/136601/download) 03/31/2020	EUA Summary (/web/20220125003500/https://www.fda.gov/media/136602/download)

Date Last Updated	Laboratory	Letter Granting Inclusion under EUA and Original Issue Date	EUA Summary
+ 06/04/2020	Nebraska Medicine Clinical Laboratory	NEcov19 RT-PCR Assay (/web/20220125003500/https://www.fda.gov/media/138624/download) * 06/04/2020	EUA Summary (/web/20220125003500/https://www.fda.gov/media/138625/download)
+ 05/22/2020	Avera Institute for Human Genetics	Avera Institute for Human Genetics SARS-CoV-2 Assay (/web/20220125003500/https://www.fda.gov/media/138331/download) 05/22/2020	EUA Summary (/web/20220125003500/https://www.fda.gov/media/138332/download)
+ 05/13/2020	One Health Laboratories, LLC	SARS-CoV-2 Real-Time RT-PCR-Test (/web/20220125003500/https://www.fda.gov/media/138062/download) 05/13/2020	EUA Summary (/web/20220125003500/https://www.fda.gov/media/138063/download)
+ 04/24/2020	Ultimate Dx Laboratory	UDX SARS-CoV-2 Molecular Assay (/web/20220125003500/https://www.fda.gov/media/137371/download) * 04/24/2020	EUA Summary (/web/20220125003500/https://www.fda.gov/media/137372/download)
+ 04/23/2020	Southwest Regional PCR Laboratory LLC. dba MicroGen DX	COVID-19 Key (/web/20220125003500/https://www.fda.gov/media/137369/download) 04/23/2020	EUA Summary (/web/20220125003500/https://www.fda.gov/media/137370/download)
+ 04/13/2020	Integrity Laboratories	SARS-CoV-2 Assay (/web/20220125003500/https://www.fda.gov/media/136941/download) 04/13/2020	EUA Summary (/web/20220125003500/https://www.fda.gov/media/136942/download)
+ 04/10/2020	Orig3n, Inc.	Orig3n 2019 Novel Coronavirus (COVID-19) Test (/web/20220125003500/https://www.fda.gov/media/136874/download) 04/10/2020	EUA Summary (/web/20220125003500/https://www.fda.gov/media/136873/download)
+ 04/10/2020	Specialty Diagnostic (SDI) Laboratories	SDI SARS-CoV-2 Assay (/web/20220125003500/https://www.fda.gov/media/136878/download) * 04/10/2020	EUA Summary (/web/20220125003500/https://www.fda.gov/media/136877/download)
+ 04/02/2020	Diagnostic Molecular Laboratory - Northwestern Medicine	SARS-Cov-2 Assay (/web/20220125003500/https://www.fda.gov/media/136658/download) * 04/02/2020	EUA Summary (/web/20220125003500/https://www.fda.gov/media/136669/download)
+ 04/02/2020	Infectious Disease Diagnostics Laboratory - Children's Hospital of Philadelphia	SARS-CoV-2 RT-PCR test (/web/20220125003500/https://www.fda.gov/media/136657/download) * 04/02/2020	EUA Summary (/web/20220125003500/https://www.fda.gov/media/136656/download)

Showing 1 to 25 of 25 entries

¹ The tests listed in this table are all molecular-based tests for COVID-19 and the authorized setting is for use in the identified laboratory that is certified under the Clinical Laboratory Improvement Amendments of 1988 (CLIA), 42 U.S.C. §263a, to perform high complexity tests.



COVID-19

We have the tools to
Fight Omicron



Vaccines & Booster



Masks



Testing

COVID-19 Testing: What You Need to Know

Updated Dec. 29, 2021

CDC has updated [isolation and quarantine](#) recommendations for the public, and is revising the CDC website to reflect these changes. These recommendations do not apply to [healthcare personnel](#) and do not supersede state, local, tribal, or territorial laws, rules, and regulations.



Free At-Home COVID-19 Tests: [Order 4 free tests now so you have them when you need them.](#) [↗](#)

Types of COVID-19 Tests

COVID-19 tests can detect either **SARS-CoV-2**, the virus that causes COVID-19, or **antibodies** that your body makes after getting COVID-19 or after getting vaccinated.

Tests for [SARS-CoV-2](#) tell you [if you have an infection](#) at the time of the test. This type of test is called a “viral” test because it looks for viral infection. Antigen or [Nucleic Acid Amplification Tests \(NAATs\)](#) are viral tests.

Tests for [antibodies](#) may tell you [if you have had a past infection](#) with the virus that causes COVID-19. Your body creates antibodies after getting infected with SARS-CoV-2 or after getting vaccinated against COVID-19. These tests are called “antibody” or “serology” tests.

Testing is very important to help reduce the spread of COVID-19.

Viral Tests

A [viral test](#) tells you if you are infected with SARS-CoV-2, the virus that causes COVID-19. There are two types of viral tests: rapid tests and laboratory tests. Viral tests use samples that come from your nose or mouth. Rapid tests can be performed in minutes and can include antigen and some NAATs. Laboratory tests can take days to complete and include RT-PCR and other types of NAATs. Some test results may need confirmatory testing.

Self-tests are rapid tests that can be taken at home or anywhere, are easy to use, and produce rapid results. COVID-19 [self-tests](#) are one of many risk-reduction measures, along with vaccination, masking, and physical distancing, that protect you and others by reducing the chances of spreading COVID-19.

Antibody Tests

An [antibody test](#) (also known as a serology test) can detect antibodies to SARS-CoV-2 in your blood. **Antibodies** are proteins that your immune system makes to help fight infection and protect you from getting sick in the future.

Antibody tests should not be used to diagnose a current infection, but may indicate if you had a past infection. Antibody tests help scientists learn about how human immune systems defend against the virus, as well as learn about population-level protection.

Antibody testing is [not currently recommended](#) to determine:

- If you have a current infection.
- If you have immunity to SARS-CoV-2 following COVID-19 vaccination.
- Whether you need to get vaccinated if you are not fully vaccinated.
- Whether you need to quarantine after a known or suspected exposure to COVID-19.

You should always discuss your test results with your healthcare provider.

Testing Tools

These chatbots ask a series of questions, and provide recommended actions and resources based on your responses.



Coronavirus Self-Checker

A tool to help you make decisions on when to seek testing and medical care.

Get Started

[About the Tool](#)



COVID-19 Viral Testing Tool

A tool to help you understand COVID-19 testing options.

Get Started

[About the Tool](#)

Video and Print Resources

Antibody Test for COVID-19

Viral Test for COVID-19

Need a COVID-19 test?

REASONS TO GET TESTED

- › If you have COVID-19 symptoms
- › After known or suspected exposure to COVID-19
- › For screening (schools, workplaces, congregate settings, etc.)
- › Before travel
- › When asked by a healthcare professional or public health official

TYPES OF VIRAL TESTS

Laboratory Test

- › Sample can either be a nasal swab or saliva
- › Results usually in 1–3 days
- › Results are reliable for people with and without symptoms
- › No follow-up test required
- › Common Example: PCR test

Rapid Test

- › Sample is usually a nasal swab
- › Results usually in 15–30 minutes
- › Results may be less reliable for people without symptoms
- › Follow-up test may be required
- › Common Example: Antigen test

ACTIONS AFTER RESULT

If positive

- › Isolate
- › Seek confirmatory, follow-up laboratory test if recommended by healthcare professional
- › Monitor your symptoms

If negative

- › If fully vaccinated, return to normal activities. Wear a mask indoors in areas of high or substantial community transmission.
- › If not fully vaccinated and have symptoms or exposure, continue to quarantine. If no symptoms or exposure return to normal activities. Take steps to get vaccinated to protect yourself and others.



Need additional help? CDC's Viral Testing Tool is an online, mobile-friendly tool that asks a series of questions, and provides recommended actions and resources based on the user's responses.

cdc.gov/coronavirus

326630-K

[JPG - 341 KB]

Need a COVID-19 Test?

Related Pages

- › [Test for Current Infection](#)
- › [Test for Past Infection](#)

Last Updated Dec. 29, 2021